



CARE INDEED HOME HEALTH

PATIENT ADMISSION PACKET

Admission, patient rights, privacy, consent, financial, emergency, OASIS, safety, plan-of-care, and signature packet. Print-ready on standard 8.5 x 11 inch letter pages.

PATIENT NAME

MEDICAL RECORD #

DATE OF BIRTH

START OF CARE DATE

PRIMARY PHYSICIAN

ADMITTING CLINICIAN

Handling note: This packet may contain protected health information when completed. Handle according to HIPAA, CMIA, and Care Indeed privacy procedures.

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Welcome Letter & Agency Identification

Dear Patient and Family,

Welcome to Care Indeed Home Health. Thank you for entrusting us with your care. We are a Medicare-certified, ACHC-accredited home health agency licensed by the California Department of Public Health (CDPH). Our agency provides skilled health care services in the comfort and privacy of your home, under the direction of your personal physician.

Our clinical team includes registered nurses, licensed vocational nurses, physical therapists, occupational therapists, speech-language pathologists, medical social workers, and certified home health aides. Every member of your care team has been credentialed, background-screened, and trained in accordance with federal and California regulations.

This packet contains important information about your rights as a patient, the services we provide, how your private health information is protected, your financial obligations, and what you can expect during your time with us. Please read each section carefully. Your admitting clinician will review these materials with you, answer any questions, and obtain your signature where required.

You may contact Care Indeed Home Health 24 hours a day, 7 days a week. Clinical and on-call support is available 24/7, including weekends and holidays.

We are committed to delivering care that is safe, effective, patient-centered, timely, efficient, and equitable. It is a privilege to care for you.

Sincerely,

_____ Administrator Care Indeed Home Health

Agency Identification Block

FIELD	DETAIL
Legal Entity Name	Care Indeed Home Health Care, Inc.
Doing Business As (DBA)	Care Indeed
HHA License Number	550004674
Medicare Certification Number (CCN)	_____
National Provider Identifier (NPI)	_____
ACHC Accreditation Number	_____
CDPH License Number	550004674
Federal Tax ID (EIN)	_____
California Business Entity Number	_____
Main Office Address	890 Santa Cruz Avenue, Suite A, Menlo Park, CA 94025
Main Office Phone	(415) 813-6888
Main Office Fax	(888) 401-7847
After-Hours / On-Call Line	(415) 813-6888 (24/7 clinical/on-call support)
Website	careindeed.com
Email (General Inquiries)	info@careindeed.com

Licensed service lines confirmed from CDPH license copy: Skilled Nursing; Home Health Aide Services; Medical Social Services; Occupational Therapy; Physical Therapy; Speech Therapy.

Key Personnel

ROLE	NAME	DIRECT PHONE	EMAIL
Administrator	Vanessa Valerio	Main office: (415) 813-6888	_____
Director of Nursing (DON)	Vanessa Valerio	Main office: (415) 813-6888	_____
Clinical Supervisor	Monserat Zapanta	Main office: (415) 813-6888	_____
HIPAA Privacy Officer	Dee Bustos	Main office: (415) 813-6888	_____
HIPAA Security Officer	Dee Bustos	Main office: (415) 813-6888	_____
Compliance Officer	Dee Bustos	Main office: (415) 813-6888	_____
Quality Assurance / QAPI Chair	Dee Bustos	Main office: (415) 813-6888	_____
Intake Coordinator	Monserat Zapanta	Main office: (415) 813-6888	_____
Billing / Financial Services	Adrian Lindain, Accounting Officer	Main office: (415) 813-6888	adianl@careindeed.com
Legal Counsel / Legal Reviewer	Edwards Law Group	_____	https://edwardslawgroup.com/

Service Area

Care Indeed Home Health is authorized to provide services in the following California counties:

_____, _____, _____, _____

(List all counties covered under license)

Accreditation & Regulatory Compliance

Care Indeed Home Health operates in compliance with:

Centers for Medicare & Medicaid Services (CMS) — Medicare Conditions of Participation (42 CFR Part 484)

- Accreditation Commission for Health Care (ACHC) — Home Health Standards

California Department of Public Health (CDPH) — Home Health Agency Licensing (Title 22, Division 5) Health Insurance Portability and Accountability Act (HIPAA) — Privacy, Security, and Breach Notification Rules California Confidentiality of Medical Information Act (CMIA) — Civil Code §56 et seq.

- California Consumer Privacy Act (CCPA/CPRA) — as applicable to non-clinical data
- OSHA / Cal-OSHA — Occupational Safety and Health Standards
- Title VI of the Civil Rights Act of 1964 — Non-Discrimination
- Section 504 of the Rehabilitation Act of 1973 — Disability Non-Discrimination
- Section 1557 of the Affordable Care Act — Non-Discrimination in Health Programs
- Americans with Disabilities Act (ADA) — Accessibility
- Patient Self-Determination Act (PSDA) — Advance Directives

California Welfare & Institutions Code §15600 et seq. — Elder and Dependent Adult Abuse Reporting

Hours Of Operation

Hours

Agency Availability	24 hours / 7 days / 365 days
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Clinical Visits	24 hours / 7 days / 365 days as ordered by physician and clinically appropriate
After-Hours On-Call	24 hours / 7 days / 365 days (RN staffed)
Holidays Observed	Clinical/on-call support available 24/7, including holidays

Care Indeed Home Health provides 24/7 clinical and on-call support. Non-urgent administrative requests are received through the main Home Health line and routed to the appropriate department for follow-up.

Patient Rights & Responsibilities

REGULATORY BASIS 42 CFR §484.50; California Health & Safety Code §1599.1 et seq.; Title 22 CCR §74743; ACHC accreditation requirements

A. PATIENT BILL OF RIGHTS

As a patient of Care Indeed Home Health, you have the right to:

Dignity & Respect

Be treated with dignity, consideration, respect, and courtesy at all times by all agency personnel. Receive care that is free from discrimination based on race, color, national origin, ancestry, ethnicity, age, sex, sexual orientation, gender identity, gender expression, marital status, religion, disability, medical condition, genetic information, source of payment, immigration status, or any other characteristic protected by federal, state, or local law. Have your cultural, psychosocial, spiritual, and personal values, beliefs, and preferences respected and accommodated to the extent possible.

Be addressed by your preferred name and pronouns.

• Informed Participation in Care

Be informed in advance about the care and treatment to be furnished, including the disciplines that will provide care, the frequency of visits, and any changes to the plan of care, before the change is made. Participate in the development and periodic revision of your individualized plan of care, including the right to establish goals of care. Be informed of anticipated outcomes of care, including realistic expected goals for improvement and any barriers to achieving those goals. Be informed of all services available from the agency, whether covered by insurance or not.

Receive a written copy of your plan of care, including the names and contact information of all providers involved in your care.

• Consent & Refusal

Give informed consent before the start of care and before any new treatment is initiated. Refuse any treatment, service, or medication after being fully informed of the expected consequences of such refusal, without being subjected to discrimination or reprisal. Revoke consent for treatment at any time. Be informed of the consequences of refusing treatment and have that refusal documented in your medical record.

• Privacy & Confidentiality

Confidentiality and privacy of all information contained in your clinical record, including the right to approve or refuse the release of your records to any individual outside the agency, except as required by law or authorized under HIPAA. Expect that all communications and records pertaining to your care will be treated as confidential, consistent with HIPAA, CMIA, and applicable California law. Privacy during care provision, including the right to have visits conducted in a private setting within your home.

Request restriction of certain disclosures of your PHI as permitted under HIPAA.

Receive care in a manner that maintains your personal privacy and bodily dignity.

• Access to Information & Records

Receive a copy of your clinical record upon request, within the time frames required by California law (no later than 15 days for paper, 5 days for electronic format). Be charged no more than the reasonable cost of copying as permitted by California Civil Code §56.11 and Health & Safety Code §123110. Request amendments or corrections to your clinical record. Receive information, upon request, relating to the agency's policies, procedures, and charges for services.

• Financial Transparency

Be informed in advance of the extent to which payment may be expected from insurance, public programs, or any other source, and the extent to which you may be responsible for any charges. Be informed, before care is initiated, of any charges for which you may be liable, including charges not covered by Medicare, Medi-Cal, or your insurance. Receive an Advance Beneficiary Notice (ABN) prior to the delivery of any service for which Medicare payment may be denied, explaining your financial liability and your options. Not be balance-billed for amounts above the Medi-Cal reimbursement rate if you are a Medi-Cal beneficiary.

- **Advance Directives**

Be informed, both orally and in writing, of your rights under California law to formulate advance directives, including a Power of Attorney for Health Care, Living Will, and POLST. Have your advance directives followed to the extent permitted by law and agency policy. Receive care regardless of whether you have executed an advance directive.

- **Safety & Freedom from Harm**

Be free from verbal, mental, sexual, and physical abuse, including injuries of unknown source, neglect, exploitation, humiliation, and misappropriation of property. Be free from the use of restraints or seclusion used as a means of coercion, discipline, convenience, or retaliation. Have your person, property, and belongings treated with respect. Receive care in a safe setting.

- **Grievances & Complaints**

Voice grievances and suggest changes in service or staff without fear of reprisal, discrimination, or interruption of services. Have grievances investigated and resolved by the agency. Be informed of the contact information for the state home health hotline, its purpose, and its hours of operation. File complaints with external agencies, including CDPH, ACHC, CMS, and the Medicare BFCC-QIO, without retaliation.

- **Transfer & Discharge**

Be informed of the agency's policies and procedures regarding transfer and discharge from services. Receive advance notice of any planned transfer or discharge, including the reasons for the transfer/discharge and your right to appeal. Not be discharged for exercising your rights, voicing grievances, or filing complaints, or for reasons of discrimination. Receive continuity of care during and after discharge, including referrals to appropriate services and providers.

- **Additional Rights**

Choose your attending physician and be informed of any limitations on that choice. Be informed of the names, professional titles, and qualifications of all individuals providing care. Receive care from personnel who have met applicable federal, state, and professional credentialing and competency requirements. Be told about any research or experimental treatment proposed as part of your care and to refuse participation without penalty. Request and receive information about the agency's ownership and control, affiliations, and any relationships with referral sources.

Be informed of your right to access auxiliary aids and services, including interpreters, at no cost.

B. PATIENT RESPONSIBILITIES

As a patient of Care Indeed Home Health, you have a responsibility to:

Communication & Information

Provide accurate, complete, and timely information about your health history, current condition, medications (including over-the-counter and supplements), allergies, past hospitalizations, and any other matters pertaining to your health. Inform your care team promptly about any changes in your condition, symptoms, medications, physician, insurance, address, phone number, emergency contacts, or advance directive status. Notify the agency if your physician makes changes to your medications or treatment plan. Communicate openly with your care team about your concerns, questions, preferences, and expectations.

Inform the agency if you do not understand your plan of care, instructions, or any information provided.

- **Plan of Care Compliance**

Follow the plan of care recommended by your physician and established in collaboration with your care team, or inform your clinician promptly if you are unable or unwilling to follow the plan. Accept the consequences of refusing treatment or not following the recommended plan of care. Take medications as prescribed and inform your clinician of any difficulties, side effects, or non-compliance. Use medical equipment and supplies as instructed and report any malfunctions or concerns.

- **Home Environment & Safety**

Provide a safe, clean, and reasonably accessible environment for care delivery, including adequate lighting, clear pathways, and a suitable work area for your clinician. Inform agency staff of any safety hazards in the home, including but not limited to: the presence of firearms or weapons, aggressive or unrestrained pets, pest infestations, structural hazards, secondhand smoke exposure, illegal activity, or the presence of individuals who may pose a threat to staff safety. Maintain utilities (electricity, water, heat/cooling) necessary for safe care delivery and the operation of medical equipment.

Ensure that a working telephone or reliable means of communication is available in the home. Address safety recommendations made by your care team to the extent reasonably possible.

- **Scheduling & Access**

Be available and present in the home for scheduled visits, or provide the agency with at least 24 hours' advance notice if you need to reschedule or cancel. Provide the agency with home access instructions (key, lockbox code, gate code, building entry procedures) if you may be unable to answer the door. Notify the agency if you will be away from home (hospitalization, vacation, temporary relocation) and provide your expected return date.

- **Financial Responsibilities**

Provide accurate and current insurance information, including copies of insurance cards. Notify the agency promptly of any changes to your insurance coverage, including new coverage, termination of coverage, or changes to a managed care plan. Meet financial obligations as agreed in the Financial Agreement (§7) and, if applicable, the Private-Pay Service Agreement (§8), in a timely manner. Notify the agency of any financial difficulties that may affect your ability to pay.

- **Respect & Cooperation**

Treat all agency staff with respect, dignity, and courtesy, free from verbal, physical, or sexual harassment, discrimination, or threats. Understand that Care Indeed Home Health has a zero-tolerance policy for abuse, threats, or harassment directed at staff, and that persistent violations may result in discharge from services after appropriate warnings and documentation. Cooperate with your care team in carrying out your plan of care. Notify the agency promptly if you have any dissatisfaction with the care or services provided, so that concerns may be addressed before they escalate.

C. STATE HOME HEALTH HOTLINE

The California Department of Public Health operates a toll-free hotline for patients to report complaints, concerns, or suspected violations related to home health services.

California Home Health Hotline	1-800-236-9747
Purpose	To receive complaints about home health agencies and to provide information about home health services
Hours	Monday – Friday, 8:00 AM – 5:00 PM (Pacific Time)
Website	www.cdph.ca.gov
Mailing Address	CDPH, Licensing & Certification Program, P.O. Box 997377, MS 3000, Sacramento, CA 95899

You may also contact the hotline if you believe your rights have been violated, you wish to report abuse or neglect by agency staff, or you have concerns about the quality of care.

☐ I have received a written copy of the Patient Rights & Responsibilities.

☐ The contents of the Patient Rights & Responsibilities have been explained to me verbally, in a language and manner I understand.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Notice Of Privacy Practices (HIPAA / CMIA)

REGULATORY BASIS 45 CFR §164.520; California Civil Code §56–56.37 (CMIA); California Civil Code §1798.100 et seq. (CCPA/CPRA, as applicable)

THIS NOTICE DESCRIBES HOW MEDICAL INFORMATION ABOUT YOU MAY BE USED AND DISCLOSED AND HOW YOU CAN GET ACCESS TO THIS INFORMATION. PLEASE REVIEW IT CAREFULLY.

A. OUR LEGAL OBLIGATIONS

Care Indeed Home Health is required by federal law (the Health Insurance Portability and Accountability Act of 1996 — "HIPAA") and California law (the Confidentiality of Medical Information Act — "CMIA") to:

Maintain the privacy of your protected health information (PHI), which includes any individually identifiable information about your health condition, the health care services you receive, or payment for those services. Provide you with this Notice of our legal duties and privacy practices with respect to your PHI. Follow the terms of the Notice currently in effect. Notify you in the event of a breach of your unsecured PHI, in compliance with the HIPAA Breach Notification Rule (45 CFR §§164.400–414) and California Civil Code §1798.82.

B. HOW WE MAY USE AND DISCLOSE YOUR PHI

• Uses and Disclosures That Do NOT Require Your Authorization

The following categories of use and disclosure are permitted or required by law without your written authorization:

1. Treatment. We will use and disclose your PHI to provide, coordinate, and manage your health care and related services. This includes sharing information among members of your care team (nurses, therapists, aides, physicians), consulting with other health care providers involved in your care, coordinating with hospitals, skilled nursing facilities, or other agencies for transfers and referrals, and communicating with your pharmacy regarding prescriptions.

Example: Your nurse shares your wound care assessment with your physician to obtain updated orders. Your physical therapist communicates with your occupational therapist regarding your mobility and functional goals.

2. Payment. We will use and disclose your PHI to obtain payment for services rendered. This includes submitting claims and supporting documentation to Medicare, Medi-Cal, private insurance companies, managed care organizations, or other third-party payers; verifying insurance coverage and eligibility; obtaining prior authorization or precertification; collecting amounts owed; and conducting utilization review activities.

Example: We submit your OASIS data and clinical documentation to Medicare to support our claim for reimbursement. We send a claim to your Medi-Cal managed care plan with diagnosis codes and dates of service.

3. Health. Care Operations We will use and disclose your PHI for our internal health care operations as necessary to run our agency and ensure quality care. This includes quality assessment and performance improvement (QAPI) activities; outcomes measurement and analysis; patient safety activities; accreditation, certification, licensing, and credentialing activities; clinical training and education (with appropriate safeguards); business planning, management, and general administration; compliance audits and fraud prevention; legal services and risk management; and business associate oversight.

Example: We review your clinical record as part of our QAPI program to identify opportunities to improve patient outcomes. Our medical records are audited during an ACHC accreditation survey.

4. Required by Law. We will disclose your PHI when required to do so by any federal, state, or local law that compels such disclosure. This includes mandatory disease and vital statistics reporting, responses to court orders and administrative orders, and compliance with workers' compensation laws.

5. Public Health Activities. We may disclose your PHI to public health authorities for the purpose of preventing or controlling disease, injury, or disability, including reporting communicable diseases to the California Department of Public Health, reporting

adverse events or product defects to the FDA, and notifying individuals who may have been exposed to a communicable disease.

6. Health Oversight Activities. We may disclose your PHI to health oversight agencies for activities authorized by law, including audits, civil, administrative, or criminal investigations, inspections, licensure and certification surveys, and other proceedings necessary for oversight of the health care system, government benefit programs, and compliance with civil rights laws. Oversight agencies include but are not limited to: CMS, ACHC, CDPH, OIG, DOJ, DHCS, and the California Attorney General.

7. Abuse and Neglect Reporting. We are required by California law (Welfare & Institutions Code §15630 et seq.) to report suspected abuse, neglect, exploitation, or abandonment of elders and dependent adults to Adult Protective Services (APS), local law enforcement, and/or the Long-Term Care Ombudsman. All Care Indeed Home Health employees are mandated reporters under California law. This disclosure does not require your authorization.

8. Judicial and Administrative Proceedings. We may disclose your PHI in response to a court order. We may also disclose PHI in response to a subpoena, discovery request, or other lawful process, provided that (a) the party seeking the information has made reasonable efforts to notify you (or your attorney), or (b) the party has obtained a qualified protective order.

9. Law Enforcement. We may disclose your PHI to law enforcement officials under limited circumstances, including: in response to a court order, warrant, subpoena, or summons; to identify or locate a suspect, fugitive, material witness, or missing person; about a suspected victim of a crime under certain limited circumstances; about a death we believe may be the result of criminal conduct; about criminal conduct occurring on our premises; and in a medical emergency when necessary to report a crime, the location of the crime or victims, or the identity, description, or location of the perpetrator.

10. Decedents. We may disclose your PHI to coroners, medical examiners, and funeral directors as necessary for them to carry out their duties. We may also disclose PHI to organ procurement organizations for the purpose of organ, eye, or tissue donation.

11. Research. We may use or disclose your PHI for research purposes if the research has been approved by an Institutional Review Board (IRB) or privacy board that has reviewed the research proposal and established protocols to ensure the privacy of your PHI. In most cases, we will ask for your specific authorization before using your PHI for research purposes.

12. To Avert a Serious Threat to Health or Safety. We may use and disclose your PHI when necessary to prevent or lessen a serious and imminent threat to your health and safety or the health and safety of the public or another person. Disclosures will be made only to someone who is able to help prevent or lessen the threat.

13. Specialized Government Functions. We may disclose your PHI for military, veterans' affairs, national security, intelligence, and protective services of the President as required by law.

14. Workers' Compensation. We may disclose your PHI as authorized and necessary to comply with workers' compensation laws or other similar programs.

15. Correctional Institutions. If you are an inmate of a correctional institution, we may disclose your PHI to the institution or its agents when necessary for your health and safety, the health and safety of others, or the administration and maintenance of safety at the institution.

16. Communication with Family, Friends, and Others Involved in. Your Care Unless you object, we may disclose your PHI to a family member, other relative, close personal friend, or any other person you identify who is involved in your care or payment for your care. We will limit the disclosure to PHI that is directly relevant to that person's involvement. If you are unable to agree or object due to incapacity or emergency, we will use our professional judgment to determine whether disclosure is in your best interest.

17. Appointment Reminders, Treatment Alternatives, and Health-Related Benefits. We may use and disclose your PHI to contact you about appointment reminders, treatment alternatives, or other health-related benefits and services that may be of interest to you.

Uses and Disclosures That REQUIRE Your Written Authorization

All uses and disclosures not described above require your signed, written authorization. Specifically, authorization is required for:

Marketing — Any communication about a product or service that encourages you to purchase or use the product or service, unless the communication is face-to-face or involves a promotional gift of nominal value. **Sale of PHI** — Any disclosure of PHI

where we receive direct or indirect remuneration from a third party in exchange for the PHI. Psychotherapy Notes — Uses or disclosures of psychotherapy notes (if any are maintained), except for certain limited treatment, payment, and operational purposes. Any other use or disclosure not described in this Notice.

You may submit an authorization using the form provided in §5 of this packet or by requesting one from our Privacy Officer.

Revocation of Authorization: You have the right to revoke any authorization you have given in writing at any time. To revoke an authorization, you must submit your request in writing to our Privacy Officer. Your revocation will be effective upon receipt, except to the extent that Care Indeed Home Health has already taken action in reliance on the authorization prior to receiving your revocation.

C. YOUR RIGHTS REGARDING YOUR PHI

Under HIPAA and California law, you have the following rights with respect to your PHI:

1. Right to Inspect and Copy (Right of Access)

You have the right to inspect and obtain a copy of your PHI that is maintained in a "designated record set," which includes your clinical/medical record and billing records. To exercise this right, you must submit a written request to our Privacy Officer.

We will respond to your request within 15 calendar days (California law) or 30 calendar days (HIPAA), whichever is shorter. If your records are maintained electronically, you may request an electronic copy in the format of your choice, if readily producible, or in another mutually agreed-upon electronic format. We may charge a reasonable, cost-based fee for copying, as permitted by California Civil Code §56.11 and Health & Safety Code §123110. The fee shall not exceed \$0.25 per page (paper) or \$0.50 per page for records copied from microfilm, plus reasonable clerical costs.

We may deny your request in limited circumstances. If we deny your request, you will receive a written explanation of the denial and information about how to request a review of the denial.

2. Right to Amend

You have the right to request that we amend your PHI if you believe it is inaccurate or incomplete. Your request must be in writing, directed to our Privacy Officer, and must include the reason you are requesting the amendment.

We will respond within 60 days of receiving your request (one 30-day extension permitted with written notice). We may deny your request if the PHI: (a) was not created by us; (b) is not part of the designated record set; (c) would not be available for your inspection; or (d) is accurate and complete. If we deny your request, we will provide a written explanation and inform you of your right to submit a written statement of disagreement, which will be included in your record.

3. Right to an Accounting of Disclosures

You have the right to request a list of certain disclosures we have made of your PHI. The accounting will include disclosures for the six-year period prior to your request (or since April 14, 2003, whichever is later) and will exclude disclosures made:

For treatment, payment, and health care operations. To you or your personal representative. Pursuant to your authorization. For national security, intelligence, or correctional purposes. As part of a limited data set. Incident to an otherwise permitted use or disclosure. We will respond within 60 days of your request (one 30-day extension permitted). The first accounting in any 12-month period is free; we may charge a reasonable, cost-based fee for additional requests.

4. Right to Request Restrictions

You have the right to request that we restrict certain uses or disclosures of your PHI for treatment, payment, or health care operations, or to persons involved in your care.

We are not required to agree to your request, except in one circumstance: if you pay for a service entirely out-of-pocket (without submitting to insurance) and you request that we not disclose PHI related to that service to your health plan for payment or operations purposes, we must comply with that restriction. If we agree to a restriction, we will comply unless the information is needed for emergency treatment. Any agreed-upon restriction may be terminated by mutual agreement or by either party with written notice.

5. Right to Request Confidential Communications

You have the right to request that we communicate with you regarding your PHI by alternative means or at alternative locations. For example, you may ask that we call you only at a specific number, use a different mailing address, or communicate via a particular email address.

We will accommodate reasonable requests. We will not ask you the reason for your request. You must specify the alternative means or location and explain how payment, if applicable, will be handled under the alternative communication method.

6. Right to a Paper Copy of This Notice

You have the right to receive a paper copy of this Notice at any time, even if you previously agreed to receive it electronically. To obtain a paper copy, contact our Privacy Officer or ask any member of your care team.

7. Right to Be Notified of a Breach

You have the right to be notified if there is a breach of your unsecured PHI. A "breach" is an unauthorized acquisition, access, use, or disclosure of PHI that compromises the security or privacy of the information. We will notify you in writing without unreasonable delay and no later than 60 days after discovery of the breach. The notification will describe the breach, the types of information involved, steps you should take to protect yourself, what we are doing to investigate and mitigate, and contact information.

D. CALIFORNIA-SPECIFIC RIGHTS (CMIA)

In addition to your rights under HIPAA, the California Confidentiality of Medical Information Act (Civil Code §56–56.37) provides the following protections:

No disclosure without authorization — Your medical information shall not be disclosed by a health care provider without your written authorization, except as specifically permitted by law. The CMIA authorization requirements are in certain respects more restrictive than HIPAA.

Mental health records — Additional protections apply to mental health records under the Lanterman-Petris-Short Act (Welfare & Institutions Code §5328).

HIV/AIDS information — Information regarding HIV testing and results is subject to additional confidentiality protections under Health & Safety Code §120975–121020.

Substance abuse treatment records — Records of substance abuse treatment are subject to federal confidentiality requirements under 42 CFR Part 2, which generally require patient consent for disclosure even for treatment, payment, and operations purposes.

Genetic information — Genetic test results are subject to additional protections under Civil Code §56.17.

Private right of action — You may bring a civil action against any person or entity that negligently or intentionally discloses your medical information in violation of the CMIA. Remedies include:

Compensatory damages

Punitive damages (for willful violations)

- Attorneys' fees and costs

Statutory damages of \$1,000 per violation (for negligent release) or \$3,000 per violation (for knowing and willful release)

- Injunctive relief

Employer restrictions — Your employer may not use your medical information for employment decisions except as specifically authorized.

E. OUR RIGHT TO CHANGE THIS NOTICE

We reserve the right to change the terms of this Notice at any time. Any revised Notice will apply to all PHI we currently maintain as well as any PHI we create or receive in the future. If we make a material change to this Notice, we will:

Make the revised Notice available upon request and at our office. Post the revised Notice on our website. Provide the revised Notice to you at your next scheduled visit or by mail.

F. COMPLAINTS

If you believe your privacy rights have been violated, you may file a complaint with:

Care Indeed Home Health — Privacy Officer Name: _____ Phone: _____ Email: _____ Address: _____

U.S. Department of Health & Human Services — Office for Civil Rights (OCR) Phone: 1-800-368-1019 TDD: 1-800-537-7697
Online: www.hhs.gov/ocr/complaints Mail: U.S. DHHS, 200 Independence Avenue SW, Room 509F, HHH Building, Washington, DC 20201

California Attorney General — Privacy Complaints Phone: (916) 210-6276 Online: oag.ca.gov/privacy/filing-complaint

You will NOT be retaliated against, penalized, or subjected to any adverse action for filing a complaint.

G. CONTACT INFORMATION

For all privacy-related questions, requests, or complaints:

HIPAA Privacy Officer	Dee Bustos
Direct Phone	Main office: (415) 813-6888
Email	_____
Mailing Address	890 Santa Cruz Avenue, Suite A, Menlo Park, CA 94025

EFFECTIVE DATE OF THIS NOTICE

July 1, 2026

Acknowledgment Of Receipt

- ☐ I acknowledge that I have received a copy of Care Indeed Home Health's Notice of Privacy Practices.
- ☐ The Notice of Privacy Practices has been explained to me verbally in a language and manner I understand.
- ☐ I have been given the opportunity to ask questions about the Notice, and my questions have been answered to my satisfaction.

If the patient refuses to sign this acknowledgment, document the refusal below:

- ☐ Patient was offered the Notice of Privacy Practices and refused to sign acknowledgment of receipt.

REASON FOR REFUSAL (IF STATED)

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Consent To Treat

REGULATORY BASIS 42 CFR §484.50(e); California Health & Safety Code §1599.1; Title 22 CCR §74659; ACHC accreditation requirements

A. PATIENT INFORMATION

FIELD	DETAIL
Patient Name	_____
Date of Birth	_____
Medical Record Number	_____
Address of Service	_____
Primary Physician	Dr. _____
Physician Phone	_____
Physician Fax	_____
Diagnosis (Primary)	_____
Start of Care Date	_____

B. SERVICES AUTHORIZED

I consent to the following home health services as ordered by my physician:

SERVICE	ORDERED (☐)	FREQUENCY (TO BE SPECIFIED IN POC)
Skilled Nursing (RN)	☐	_____
Skilled Nursing (LVN)	☐	_____
Physical Therapy (PT)	☐	_____
Occupational Therapy (OT)	☐	_____
Speech-Language Pathology (SLP)	☐	_____
Medical Social Work (MSW)	☐	_____
Home Health Aide (HHA)	☐	_____
Dietitian / Nutritional Counseling	☐	_____
Infusion Therapy / IV Services	☐	_____
Wound Care / Ostomy Care	☐	_____
Other: _____	☐	_____

C. INFORMED CONSENT STATEMENTS

By signing below, I, _____ (Patient Name), or my authorized representative, acknowledge and consent to the following:

Nature of Services. I consent to receive home health services from Care Indeed Home Health as described above and as further specified in my individualized plan of care (POC). I understand that specific services, their frequency, and duration will be determined by my physician in collaboration with the clinical team and will be documented in my POC.

Physician Orders. I understand that all skilled services are provided under the orders of my attending physician and that my plan of care must be signed by my physician. I understand that Care Indeed Home Health cannot provide services without valid physician orders.

Right to Participate. I understand that I have the right to participate in the development of my plan of care, to set goals for my care, and to be informed of any changes to my plan before they are made.

Right to Refuse. I understand that I have the right to refuse any service, treatment, or medication at any time. I understand that if I refuse services, the potential consequences of my refusal will be explained to me and documented in my medical record.

Qualified Personnel. I understand that all services will be provided by qualified, credentialed personnel who are employees or contracted providers of Care Indeed Home Health. All personnel have undergone criminal background screening, verification of licensure/certification, health screening (including TB clearance), and competency evaluation in compliance with federal and California regulations.

Supervision. I understand that home health aides are supervised by a registered nurse; licensed vocational nurses practice under the direction of a registered nurse or physician; and therapy assistants practice under the supervision of a licensed therapist, all in accordance with California scope of practice laws.

Students and Trainees. I understand that Care Indeed Home Health may participate in clinical education programs. If a student or trainee will participate in my care, I will be notified in advance and may refuse to have a student involved in my care without penalty.

Communication with Providers. I consent to the exchange of clinical information between Care Indeed Home Health, my attending physician(s), my specialists, my pharmacist, my hospital/facility providers, my insurance carrier(s), and other health care providers involved in my care, as necessary for treatment, payment, and health care operations, and as described in the Notice of Privacy Practices (§3).

Photographs / Wound Documentation. I consent to the taking of clinical photographs of wounds, skin conditions, or other physical findings for the purpose of clinical documentation, care coordination, and treatment monitoring. These photographs become part of my protected medical record and are subject to the same confidentiality protections.

☐ I consent to clinical photography for documentation purposes. ☐ I decline clinical photography. (Refusal documented)

Laboratory Specimens. If ordered by my physician, I consent to the collection, handling, and transport of laboratory specimens (blood, urine, wound cultures, etc.) by agency personnel, and to the sending of such specimens to a certified laboratory for analysis.

Duration of Consent. This consent shall remain in effect for the duration of my episode of care with Care Indeed Home Health, including any recertification periods, unless I revoke it in writing. I may revoke this consent at any time by notifying Care Indeed Home Health in writing. Revocation will not affect any actions already taken in reliance on this consent.

No Guarantees. I understand that no guarantees have been made regarding the results of services provided by Care Indeed Home Health. The goal of home health services is to improve or maintain my health status and functional abilities, but outcomes depend on many factors including my condition, my compliance with the plan of care, and my home environment.

D. SIGNATURES

Patient / Authorized Representative

IF PATIENT IS UNABLE TO SIGN, STATE REASON

Admitting Clinician / Witness

TITLE / CREDENTIALS

☐ An interpreter was used during the consent process.

LANGUAGE

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Authorization For Use And Disclosure Of Protected Health Information (HIPAA ROI)

REGULATORY BASIS 45 CFR §164.508; California Civil Code §56.11

Standalone authorization: execute this HIPAA ROI only when a specific disclosure is requested. It is voluntary and not bundled into the master admission signature.

IMPORTANT: This authorization is entirely voluntary. You are not required to sign it. Your refusal to sign will NOT affect your ability to receive treatment, obtain insurance coverage, receive payment, or be eligible for benefits.

A. PATIENT INFORMATION

FIELD	DETAIL
Patient Name	_____
Date of Birth	_____
Medical Record Number	_____
Address	_____
Phone	_____

B. PERSON / ORGANIZATION AUTHORIZED TO MAKE THE DISCLOSURE

FIELD	DETAIL
Name	Care Indeed Home Health
Address	_____
Phone	_____
Fax	_____

C. PERSON / ORGANIZATION AUTHORIZED TO RECEIVE THE INFORMATION

FIELD	DETAIL
Name	_____
Organization	_____
Address	_____
Phone	_____
Fax	_____
Secure Email	_____

D. SPECIFIC INFORMATION TO BE DISCLOSED

Check all that apply:

- ☐ Complete Medical / Clinical Record
- ☐ History & Physical Examination
- ☐ Discharge Summary
- ☐ Plan of Care / 485
- ☐ OASIS Assessment Data
- ☐ Skilled Nursing Notes / Visit Notes
- ☐ Therapy Notes (PT/OT/SLP)
- ☐ Medication List / Medication Administration Record
- ☐ Laboratory / Diagnostic Test Results
- ☐ Wound Care Records / Photographs
- ☐ Physician Orders
- ☐ Progress Notes
- ☐ Billing Records / Claims Information
- ☐ Immunization Records
- ☐ Transfer / Referral Documentation
- ☐ Mental Health Records (requires separate authorization per Lanterman-Petris-Short Act)
- ☐ Substance Abuse Treatment Records (requires separate authorization per 42 CFR Part 2)
- ☐ HIV/AIDS-Related Information (requires specific authorization per CA H&S Code §120975)
- ☐ Genetic Information (requires specific authorization per CA Civil Code §56.17)
- ☐ Other (specify): _____

DATE RANGE OF RECORDS

From _____ To _____

E. PURPOSE OF THE DISCLOSURE

Check all that apply:

- ☐ Continuity of Care / Transfer to Another Provider
- ☐ Consultation with Specialist
- ☐ Insurance / Benefits / Coverage Determination
- ☐ Legal Proceedings

- ☐ Disability Determination
- ☐ Personal / Patient Request
- ☐ Family Member / Caregiver Communication
- ☐ Other (specify): _____

F. EXPIRATION

This authorization shall expire on:

- ☐ Specific Date: _____
- ☐ Upon occurrence of the following event: _____
- ☐ If no date or event is specified, this authorization shall expire one (1) year from the date of signature.

G. YOUR RIGHTS AND IMPORTANT INFORMATION

Please read the following carefully before signing:

Voluntary. This authorization is voluntary. Care Indeed Home Health will not condition treatment, payment, enrollment in a health plan, or eligibility for benefits on your signing this authorization.

Right to Revoke. You may revoke this authorization at any time by submitting a written revocation to our Privacy Officer. Revocation will be effective upon receipt, except to the extent that Care Indeed Home Health has already acted in reliance on the authorization prior to receiving your revocation.

Re-Disclosure Warning. Once your PHI is disclosed pursuant to this authorization, it may be subject to re-disclosure by the recipient and may no longer be protected by HIPAA or the CMIA. However, California law prohibits the recipient from further disclosing your medical information without your additional authorization, except as specifically permitted by law (Civil Code §56.13).

Copy. You are entitled to a copy of this signed authorization.

Fees. Care Indeed Home Health may charge a reasonable, cost-based fee for producing copies of records, consistent with California law.

Sensitive Information. If you are authorizing the release of particularly sensitive information (mental health, substance abuse, HIV/AIDS, genetic information), additional legal protections may apply, and separate authorization forms may be required.

Minors. If the patient is a minor, the signature of a parent or legal guardian is required except as otherwise permitted by California law.

H. SIGNATURES

Patient / Authorized Representative

SIGNATURE

_____ Date: _____ Time: _____

PRINT NAME

RELATIONSHIP TO PATIENT (IF REPRESENTATIVE)

LEGAL AUTHORITY (IF APPLICABLE)

Witness

SIGNATURE

_____ Date: _____

PRINT NAME / TITLE

Advance Directive Information & Acknowledgment

REGULATORY BASIS 42 CFR §484.50; 42 CFR §489.102; Patient Self-Determination Act (42 USC §1395cc(f)); California Probate Code §4600–4806; California Health & Safety Code §7185–7195 (Natural Death Act); Title 22 CCR §74667

A. WHAT ARE ADVANCE DIRECTIVES?

Advance directives are legal documents that allow you to express your wishes about future medical treatment and to designate someone to make health care decisions on your behalf if you become unable to do so. Under California law, the following types of advance directives are recognized:

1. Advance Health Care Directive (AHCD) A written document that may include both of the following:

Power of Attorney for Health Care — Designates another person (your "agent" or "health care proxy") to make health care decisions on your behalf when you are unable to make them yourself. Your agent has the authority to make any health care decision you could make, including decisions about life-sustaining treatment, unless you specifically limit that authority.

Individual Health Care Instructions (Living Will) — Written instructions that express your wishes regarding specific types of health care, including your desires about life-sustaining treatment, artificial nutrition and hydration, pain management, organ donation, and disposition of remains.

2. Physician Orders for Life-Sustaining Treatment (POLST) A medical order (not merely a directive) completed by a health care provider in consultation with you (or your representative). The POLST form specifies the types of life-sustaining treatment you wish to receive or decline, including CPR, intubation/mechanical ventilation, antibiotics, artificial nutrition, and comfort measures. The POLST is intended for individuals with serious illness or advanced frailty and is recognized by emergency medical personnel.

3. Do Not Resuscitate (DNR) Order A physician order that instructs health care providers and emergency medical personnel not to initiate cardiopulmonary resuscitation (CPR) in the event of cardiac or respiratory arrest. In California, a valid pre-hospital DNR must be on the state-approved POLST or equivalent form to be honored by paramedics and emergency responders.

4. Mental Health Advance Directive A document that allows you to express your wishes about mental health treatment, including hospitalization, medication, and electroconvulsive therapy, and to appoint an agent for mental health decisions.

B. IMPORTANT INFORMATION ABOUT ADVANCE DIRECTIVES

You are NOT required to have an advance directive to receive services from Care Indeed Home Health. We will provide the same quality of care regardless of whether you have executed an advance directive.

California law protects your right to make your own medical decisions. If you lose the capacity to make decisions and do NOT have an advance directive, California's health care decisions law (Probate Code §4714) establishes a priority list of individuals authorized to make decisions on your behalf:

Spouse or registered domestic partner

- Adult child
- Parent
- Adult sibling
- Any other adult relative or friend who is familiar with your values

How to create an advance directive: You may use the statutory form provided in California Probate Code §4701, or any other substantially compliant form. The document must be:

Signed by you (or at your direction) Either notarized or signed by two adult witnesses who are present at the same time (with restrictions on who may serve as a witness) You may change or revoke your advance directive at any time by executing a new

directive, by written revocation, or by verbal revocation communicated to your health care provider.

Copies are as valid as originals under California law (Probate Code §4675).

C. CARE INDEED HOME HEALTH — ADVANCE DIRECTIVE POLICY

At the time of admission, your admitting clinician will:

Inform you of your right to formulate advance directives. Ask whether you have existing advance directives. Request copies of any existing directives, POLST, or DNR orders for inclusion in your clinical record. Document your advance directive status in your medical record and plan of care. Offer you assistance and resource materials if you wish to create advance directives. If you have advance directives:

A copy will be placed in your clinical record. Your care team will be informed of the contents. Your directives will be honored to the extent permitted by law and consistent with professional standards. If a specific provision of your directive cannot be honored (e.g., due to a conflict with agency policy or professional ethics), you will be informed and the agency will assist with transfer to a provider that can comply. If you do NOT have advance directives:

Your care will not be affected in any way. You will be offered information and resources to assist you if you choose to create directives in the future. Your advance directive status will be reassessed at each recertification period (at least every 60 days). In a medical emergency:

If you have a valid POLST or DNR on file, agency staff will follow those orders. If no directive or POLST is on file, agency staff will initiate standard emergency procedures (including calling 911 and beginning CPR as appropriate) until emergency medical services arrive. Agency staff are not authorized to interpret or enforce advance directives in the field unless the directive is a valid POLST or pre-hospital DNR recognized under California law. Education and Resources:

Your admitting clinician will provide you with written advance directive information appropriate to your language and literacy level. Referrals to legal aid services, hospital social workers, or community organizations that assist with advance directive completion are available upon request. The California Secretary of State maintains an optional Advance Health Care Directive Registry.

D. ACKNOWLEDGMENT

Please check all that apply:

- ☐ I have been informed, both verbally and in writing, of my right to formulate advance directives under California and federal law.
- ☐ I have been informed of Care Indeed Home Health's policies regarding advance directives, including the agency's duty to honor valid directives and the circumstances under which a directive may not be implemented.
- ☐ I DO have an existing advance directive:
 - ☐ Advance Health Care Directive (AHCD) — copy provided
 - ☐ POLST — copy provided
 - ☐ DNR Order — copy provided
 - ☐ Mental Health Advance Directive — copy provided
 - ☐ Other: _____ — copy provided
- ☐ I DO NOT currently have an advance directive.
- ☐ I would like more information / assistance with creating one.

- ☐ I decline further discussion about advance directives at this time.
- ☐ I have been offered assistance in obtaining advance directive forms and/or referrals to resources.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Financial Agreement & Payment Terms

REGULATORY BASIS 42 CFR §484.50(a)(3)–(4); 42 CFR §489.20(a); California Health & Safety Code §1599.1(g); Title 22 CCR §74743(a)(10); Medicare Benefit Policy Manual Ch. 7; Medi-Cal Provider Manual; ACHC accreditation requirements

A. PAYER SOURCE IDENTIFICATION

Check all applicable payer sources:

PRIORITY	PAYER	POLICY/ID NUMBER	GROUP NUMBER	EFFECTIVE DATE
Primary	☐ Medicare Part A (Traditional)	_____	N/A	_____
Primary	☐ Medicare Advantage Plan: _____	_____	_____	_____
Secondary	☐ Medi-Cal (Fee-for-Service)	_____	N/A	_____
Secondary	☐ Medi-Cal Managed Care Plan: _____	_____	_____	_____
☐ Private Insurance: _____	_____	_____	_____	_____
☐ Workers' Compensation: _____	_____	_____	_____	_____
☐ VA / CHAMPVA / TRICARE	_____	_____	_____	_____
☐ Private Pay (see §8)	N/A	N/A	_____	_____
☐ Other: _____	_____	_____	_____	_____

B. MEDICARE PATIENTS — FINANCIAL RIGHTS AND OBLIGATIONS

If you are a Medicare beneficiary receiving home health services under the Medicare home health benefit:

No Cost-Sharing for Covered Services. Medicare-covered home health services have no deductible and no co-payment. You will not be billed any amount for services covered under the Medicare home health benefit, including skilled nursing, physical therapy, occupational therapy, speech-language pathology, medical social work, and home health aide services.

DME Exception. Durable medical equipment (DME) furnished under a separate DME benefit may be subject to the standard Medicare Part B 20% co-insurance and annual deductible.

Advance Beneficiary Notice (ABN). If Care Indeed Home Health believes that Medicare may not cover a specific service or supply, we will provide you with a written Advance Beneficiary Notice of Noncoverage (ABN — CMS Form R-131) before the service is provided. The ABN will:

Identify the specific item or service that may not be covered. State the reason we expect Medicare may deny payment. Provide you with an estimated cost. Give you the following options: Option 1: Receive the service and accept financial responsibility if Medicare does not pay. You may ask Medicare to pay by filing a claim, and if denied, you may appeal. Option 2: Receive the service and accept financial responsibility. Medicare will not be billed, and you will not have appeal rights. Option 3: Decline the service. You will not be charged, and Medicare will not be billed.

Notice of Medicare Non-Coverage. Before your Medicare-covered services end, you will receive a Notice of Medicare Non-Coverage explaining the date services will end and your right to request an expedited review (appeal) from the Quality Improvement Organization (QIO — Livanta).

Medicare Summary Notice (MSN). You will receive a Medicare Summary Notice from Medicare every three months showing what services were billed on your behalf, what Medicare paid, and what (if anything) you may owe.

No Inducements. Care Indeed Home Health does not offer or provide any gifts, payments, or other inducements to encourage referrals or the use of services, in compliance with the Medicare Anti-Kickback Statute (42 USC §1320a-7b(b)) and the Stark Law (42 USC §1395nn).

C. MEDI-CAL (MEDICAID) PATIENTS — FINANCIAL PROTECTIONS

If you are a Medi-Cal beneficiary:

No Balance Billing. Care Indeed Home Health will not bill you for any amount above the Medi-Cal reimbursement rate for covered services. This prohibition is absolute and non-negotiable under federal and California law (42 USC §1396a(a)(25)(C); Welfare & Institutions Code §14019.3).

Share of Cost. If you have a Medi-Cal "share of cost," you are responsible for meeting your monthly share of cost obligation before Medi-Cal will cover services. Care Indeed Home Health will assist you in understanding your share of cost and may collect applicable share of cost amounts.

Co-payments. Medi-Cal co-payments, if applicable to your benefit category, will be collected in accordance with DHCS guidelines. You will be informed of any applicable co-payment before services are rendered.

Non-Covered Services. If a service is not covered by Medi-Cal, you will be informed in advance and will not be held financially responsible unless you provide written informed consent agreeing to accept the charge.

Dual-Eligible Patients (Medicare + Medi-Cal). If you are enrolled in both Medicare and Medi-Cal, Medicare is your primary payer. Medi-Cal may pay for services not covered by Medicare, including Medi-Cal co-payments on Medicare services. You should not receive any bill for covered services.

D. PRIVATE INSURANCE PATIENTS — FINANCIAL OBLIGATIONS

If your services are covered by private insurance, a Medicare Advantage plan, or an HMO/managed care plan:

Courtesy Billing. Care Indeed Home Health will bill your insurance carrier directly as a courtesy. However, you are ultimately responsible for understanding your plan's benefits, limitations, exclusions, co-payments, deductibles, co-insurance, and prior authorization requirements.

Verification of Benefits. We will make reasonable efforts to verify your insurance benefits and obtain necessary authorizations before or at the start of care. However, verification of benefits is not a guarantee of payment.

Prior Authorization. Some insurance plans require prior authorization for home health services. You are responsible for notifying us of any authorization requirements you are aware of. If your plan denies authorization, we will notify you before discontinuing services.

Co-Payments, Deductibles, and Co-Insurance. You are responsible for any co-payments, deductibles, and co-insurance amounts required by your plan. These amounts may be collected at the time of service or billed to you.

Non-Covered Services. If your insurance does not cover a specific service, you will be informed in advance and given the option to:

Decline the service at no charge. Receive the service and accept financial responsibility. Timely Payment. Any balance not paid by your insurance carrier within 60 days of initial claim submission will be investigated by our billing department. If the claim is denied or partially paid and you are responsible for the remaining balance, you will be billed.

Out-of-Network. If Care Indeed Home Health is not in your insurance plan's network, you may be responsible for higher out-of-pocket costs. We will advise you of our network status to the extent known.

E. ASSIGNMENT OF BENEFITS

I hereby assign and authorize payment of insurance benefits, including Medicare, Medi-Cal, and private insurance benefits, directly to Care Indeed Home Health for services rendered. I understand that I am financially responsible for any charges not covered by my insurance, except as limited by applicable law (including the Medicare and Medi-Cal balance billing prohibitions described above).

F. AUTHORIZATION TO RELEASE INFORMATION FOR BILLING

I authorize Care Indeed Home Health to release any medical or other information necessary to process claims for payment to my insurance carrier(s), Medicare, Medi-Cal, or other third-party payer, including diagnosis codes, procedure codes, clinical documentation, and any other information required for claims processing, utilization review, or audit purposes.

G. GENERAL FINANCIAL TERMS

Statements. You will receive periodic statements detailing charges, payments, adjustments, and any outstanding balance.

Questions About Bills. If you have questions about a bill or charge, please contact our billing department at _____. We will investigate and respond within 30 days of your inquiry.

Dispute Resolution. If you believe a charge is incorrect, you have the right to dispute it in writing within 60 days of receiving the statement. We will investigate the dispute and respond in writing.

Collections. Accounts with balances remaining unpaid for more than 90 days after final adjudication by all payers and exhaustion of appeals may be referred for collection. You will receive at least two written notices before any account is referred for collection.

Charity Care / Financial Hardship. If you are experiencing financial hardship, please notify our billing department. Care Indeed Home Health will evaluate eligibility for charity care, payment plans, or reduced fees on a case-by-case basis. Financial hardship will not result in denial or reduction of clinically necessary services for Medicare patients.

Non-Discrimination. Care Indeed Home Health does not deny or reduce services based on a patient's inability to pay for services covered under federal or state programs. Services will not be conditioned on the execution of a financial guarantee by a third party.

H. SIGNATURES

Patient / Authorized Representative

I have read and understand the financial terms described above. I agree to the assignment of benefits, the release of information for billing purposes, and the financial obligations described for my payer category.

Agency Representative

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Private-Pay Service Agreement

REGULATORY BASIS California Civil Code §1750 et seq. (CLRA); Business & Professions Code §17200 et seq. (UCL); California Health & Safety Code §1599.1(g)

Conditional private-pay section: render/sign this section only when the patient or responsible party is private pay or a legally approved private-pay addendum applies. Rates, late fees, cancellation, and collection terms require final legal approval before production use.

This section applies ONLY to patients receiving services on a self-pay/private-pay basis, either in whole or for services not covered by insurance.

A. PARTIES

Provider: Care Indeed Home Health Care, Inc. Address: _____ Phone: _____ License #: _____

Patient/Responsible Party: _____ Address: _____ Phone: _____ Email: _____

B. SCOPE OF SERVICES AND RATES

SERVICE	DESCRIPTION	HOURLY RATE	MINIMUM HOURS/VISIT	ESTIMATED HOURS/WEEK
Skilled Nursing (RN)	_____	\$ _____	_____	_____
Skilled Nursing (LVN)	_____	\$ _____	_____	_____
Physical Therapy (PT)	_____	\$ _____ per visit	N/A	_____
Occupational Therapy (OT)	_____	\$ _____ per visit	N/A	_____
Speech-Language Pathology (SLP)	_____	\$ _____ per visit	N/A	_____
Medical Social Work (MSW)	_____	\$ _____ per visit	N/A	_____
Home Health Aide (HHA)	_____	\$ _____	_____	_____
Other: _____	_____	\$ _____	_____	_____

ESTIMATED WEEKLY COST

\$ _____ Estimated Monthly Cost: \$ _____

Note: Estimates are based on projected hours/visits and may vary depending on actual utilization. You will be billed based on actual services provided.

C. ADDITIONAL CHARGES

The following additional charges may apply:

ITEM	CHARGE
Medical supplies / wound care supplies	At cost + ____%
Mileage (if applicable)	\$ _____ per mile
Holiday visit surcharge	____% of standard rate

ITEM	CHARGE
Weekend / after-hours surcharge (if applicable)	___ % of standard rate
Rush scheduling (< 24 hrs notice)	\$ _____
Administrative fee for records copying	\$ _____ per page

D. BILLING AND PAYMENT TERMS

- Billing Cycle. Services will be billed: ☐ Weekly ☐ Bi-weekly ☐ Monthly

Payment Due Date. Payment is due within fifteen (15) calendar days of the invoice date.

Accepted Payment Methods:

- ☐ Personal Check
- ☐ Cashier's Check / Money Order
- ☐ Credit Card (Visa / MasterCard / AmEx / Discover)
- ☐ ACH / Electronic Bank Transfer
- ☐ Other: _____

Deposit. A deposit of \$ _____ ☐ is required / ☐ is not required. If required, the deposit will be applied to the final billing period or refunded within 30 days of service termination, less any outstanding balance.

Late Payment. A late fee of 1.5% per month (or the maximum permitted under California Civil Code §1671, whichever is less) may be applied to balances overdue by more than thirty (30) days.

Returned Checks. A fee of \$ _____ (not to exceed the maximum permitted under California Civil Code §1719) will be charged for any check returned for insufficient funds.

Collection. Accounts remaining unpaid for more than 90 days may be referred for collection. You will be responsible for all reasonable collection costs, including attorneys' fees, to the extent permitted by law.

E. RATE CHANGES

Care Indeed Home Health reserves the right to adjust service rates. You will receive thirty (30) days' written notice before any rate change takes effect. If you do not agree to the new rates, you may terminate this agreement by providing written notice within the 30-day notification period.

F. CANCELLATION POLICY

Scheduled Visit Cancellation. Visits cancelled with less than 24 hours' notice may be billed at:

- ☐ Full visit rate
- ☐ 50% of visit rate
- ☐ Flat fee of \$ _____

No-Show. If the patient is not available for a scheduled visit without advance notification, the visit may be billed at the full visit rate.

Agency-Initiated Cancellation. If Care Indeed Home Health must cancel or reschedule a visit (e.g., due to staffing, weather, or emergency), you will be notified as soon as possible and no charge will apply. We will make reasonable efforts to arrange a

replacement visit.

Chronic Cancellations. Repeated cancellations or no-shows (more than ____ in a 30-day period) may be grounds for a care conference to discuss continued services.

G. TERM AND TERMINATION

EFFECTIVE DATE. THIS AGREEMENT IS EFFECTIVE ON

Duration. This agreement shall remain in effect until terminated by either party.

Patient-Initiated Termination. You may terminate this agreement at any time for any reason by providing fourteen (14) days' written notice to Care Indeed Home Health. You are responsible for payment for all services rendered through the effective date of termination.

Agency-Initiated Termination. Care Indeed Home Health may terminate this agreement with fourteen (14) days' written notice for the following reasons:

Non-payment of amounts due after reasonable collection efforts. Persistent safety concerns in the home that are not remediated. Verbal, physical, or sexual abuse or threats directed at staff. The patient's needs exceed the agency's scope of services. Other just cause. Continuity of Care. Upon termination, Care Indeed Home Health will:

Provide referrals to alternative home health agencies or providers. Transfer clinical records to the new provider upon your written authorization. Ensure no gap in critical services to the extent possible.

H. CALIFORNIA CONSUMER DISCLOSURES

Itemized Statements. You have the right to receive an itemized statement of all charges upon request at any time during or after the service period.

Dispute Rights. You have the right to dispute any charge in writing within sixty (60) days of receipt of the invoice. Care Indeed Home Health will investigate and provide a written response within thirty (30) days.

Consumer Protection. This agreement does not waive any rights afforded to you under:

The California Consumer Legal Remedies Act (Civil Code §1750 et seq.) The California Unfair Competition Law (Business & Professions Code §17200 et seq.) The California False Advertising Law (Business & Professions Code §17500 et seq.)

- Any other applicable consumer protection statute

No Hidden Fees. All fees and charges have been disclosed in this agreement. No additional fees will be assessed without prior written notice.

Governing Law. This agreement shall be governed by the laws of the State of California.

I. SIGNATURES

Patient / Responsible Party

I have read and understand the terms of this Private-Pay Service Agreement. I agree to the scope of services, rates, billing terms, cancellation policy, and termination provisions described herein.

SIGNATURE

_____ Date: _____

PRINT NAME

RELATIONSHIP TO PATIENT (IF RESPONSIBLE PARTY IS NOT THE PATIENT)

Agency Representative

SIGNATURE

_____ Date: _____

PRINT NAME / TITLE

A signed copy of this agreement will be provided to the patient/responsible party.

Emergency Preparedness Plan Summary

REGULATORY BASIS 42 CFR §484.102; California Health & Safety Code §1797 et seq.; Title 22 CCR §74665; ACHC Emergency Management Standards

A. AGENCY EMERGENCY PREPAREDNESS PROGRAM

Care Indeed Home Health maintains a comprehensive, all-hazards Emergency Preparedness Plan (EPP) in compliance with the CMS Emergency Preparedness Rule (42 CFR §484.102). Our plan is reviewed and updated at least annually, and agency-wide exercises (tabletop or functional) are conducted at least twice per year.

OUR EMERGENCY PREPAREDNESS PLAN ADDRESSES THE FOLLOWING HAZARDS AND SCENARIOS

Natural Disasters

Earthquakes

- Wildfires and wildfire smoke events
- Flooding and mudslides
- Extreme heat events
- Severe storms and high-wind events

Tsunami (coastal service areas, if applicable)

- Technological / Human-Caused Events

Power outages and rolling blackouts (including Public Safety Power Shutoffs — PSPS)

- Water supply disruption or contamination
- Cyber attacks on information systems
- Hazardous material releases
- Active threat / violence events
- Public Health Emergencies

Pandemics and epidemic outbreaks

- Emerging infectious diseases
- Mass casualty events

B. PATIENT-SPECIFIC EMERGENCY INFORMATION

The following information will be maintained in your clinical record and used during emergency response:

FIELD	DETAIL
Patient Name	_____
Address of Service	_____
Primary Phone	_____
Alternate Phone	_____
Primary Emergency Contact	Name: _____ Relationship: _____ Phone: _____
Secondary Emergency Contact	Name: _____ Relationship: _____ Phone: _____

FIELD	DETAIL
Physician	Dr. _____ Phone: _____
Pharmacy	_____ Phone: _____
Hospital Preference	_____
Evacuation Location (if planned)	_____
Life-Sustaining Equipment in Home	☐ Oxygen ☐ Ventilator ☐ Suction ☐ Feeding Pump ☐ IV Pump ☐ Dialysis ☐ Other: _____
Mobility Status	☐ Independent ☐ Needs Assistance ☐ Wheelchair ☐ Bedbound
Special Needs During Evacuation	_____
Registered with Utility Company (for life-sustaining equipment)	☐ Yes ☐ No ☐ N/A
Registered with County OES / 211	☐ Yes ☐ No
Pets in Home	☐ Yes (type/number: _____) ☐ No

C. DURING AN EMERGENCY — AGENCY ACTIONS

Notification. Our designated emergency coordinator will activate the emergency communication plan. Staff will attempt to contact you by phone, text, and/or email to assess your safety and immediate needs.

Prioritization. Patients will be triaged based on acuity and risk:

Priority 1 (Critical): Patients dependent on life-sustaining equipment (oxygen, ventilator, dialysis), patients on IV medications, patients with acute instability. Priority 2 (High): Patients with complex wound care, patients living alone with limited mobility, patients requiring daily skilled nursing visits. Priority 3 (Standard): All other patients. Welfare Checks. If you cannot be reached by phone, and it is safe for staff to travel, an in-person welfare check will be attempted for Priority 1 and 2 patients.

Service Modifications. During a declared emergency, service delivery may be temporarily modified:

Visit frequency may be adjusted based on your priority level and staff availability. Telehealth visits may be substituted for in-person visits when clinically appropriate and feasible. Alternative care arrangements (transfer to hospital, skilled nursing facility, or shelter medical unit) may be coordinated if home-based care becomes unsafe or impossible. Record Access. Clinical records are maintained in a cloud-based electronic health record (EHR) system with redundant backup and disaster recovery capability, ensuring continuity of access during local infrastructure disruptions.

Coordination. We maintain agreements with community partners, emergency management agencies, and local health departments for coordinated response during major disasters.

D. PATIENT / FAMILY RESPONSIBILITIES DURING AN EMERGENCY

72-Hour Emergency Kit. Maintain a ready emergency supply kit containing:

At least 72 hours' supply of all medications (clearly labeled) At least 72 hours' supply of water (1 gallon per person per day)

- Non-perishable food and eating utensils
- Flashlight, batteries, and battery-operated or hand-crank radio
- First aid kit

Copies of important documents (insurance cards, ID, medication list, advance directives)

- Personal care items
- Charged cell phone and portable charger

Cash (small bills)

- Extra eyeglasses, hearing aid batteries, and other essential personal items

Supplies for any medical devices (extra tubing, batteries, etc.) Pet supplies (if applicable) Medication List. Keep a current medication list (including name, dose, frequency, prescriber, and pharmacy) in an accessible location and provide a copy to your emergency contacts.

Emergency Contacts. Ensure your emergency contact information with Care Indeed Home Health is current. Notify us immediately of any changes.

Evacuation Plan. Know your evacuation routes and have a pre-arranged evacuation destination. Inform Care Indeed Home Health of your evacuation location if you relocate during an emergency.

Life-Sustaining Equipment. If you use electrically powered life-sustaining equipment:

Register with your utility company (PG&E, SCE, SDG&E) as a medical baseline customer for priority restoration. Register with your county Office of Emergency Services (OES) or call 2-1-1 for disaster assistance programs. Maintain a backup power source (battery backup, portable generator) and know how to use it. Have a manual alternative available (e.g., manual suction, hand ventilation bag). Communication. During an emergency:

Answer calls from Care Indeed Home Health or return messages promptly. If you evacuate, contact the agency as soon as possible with your new location and phone number. Designate an out-of-area emergency contact who can relay your status if local communication is disrupted. Text messaging may be more reliable than phone calls during widespread emergencies.

E. KEY EMERGENCY NUMBERS

CONTACT	PHONE NUMBER
Emergency (Police / Fire / EMS)	911
Poison Control	1-800-222-1222
Care Indeed Home Health — Main Office	(415) 813-6888 (ask for Administrator, DON, or Compliance Officer)
Care Indeed Home Health — After-Hours On-Call	(415) 813-6888 (ask for Administrator, DON, or Compliance Officer)
County Office of Emergency Services	_____
2-1-1 (Community Resources / Disaster Assistance)	2-1-1
Utility Company Emergency Line	_____
American Red Cross	1-800-733-2767
FEMA	1-800-621-3362
Patient's Physician	Dr. _____ : _____
Patient's Pharmacy	_____

F. ACKNOWLEDGMENT

- ☐ I have received and understand the Emergency Preparedness Plan Summary.
- ☐ I have been encouraged to prepare a 72-hour emergency kit and evacuation plan.
- ☐ My emergency contact information above is accurate and current.
- ☐ I have been informed of the need to register with my utility company if I use life-sustaining equipment. (If applicable)

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Abuse, Neglect & Exploitation Reporting Notice

REGULATORY BASIS 42 CFR §484.12(b)(3); California Welfare & Institutions Code §15600–15660 (Elder Abuse and Dependent Adult Civil Protection Act); California Penal Code §368; Title 22 CCR §74743; ACHC accreditation requirements

A. ZERO-TOLERANCE POLICY

Care Indeed Home Health maintains an absolute zero-tolerance policy for abuse, neglect, exploitation, and misappropriation of patient property. This policy applies to all agency employees, contractors, volunteers, students, and any person providing services on behalf of the agency.

Any substantiated finding of abuse, neglect, or exploitation will result in immediate termination of the responsible individual, reporting to the appropriate licensing board, reporting to law enforcement, and reporting to Adult Protective Services, CDPH, and CMS as required.

B. DEFINITIONS

Abuse (Physical): The intentional or reckless use of physical force or physical behavior directed against a patient that results in, or has the potential to result in, bodily injury, physical pain, or impairment. Includes but is not limited to: hitting, slapping, pushing, kicking, pinching, burning, improper use of physical restraints, and forced feeding.

Abuse (Sexual): Non-consensual sexual contact of any type, including but not limited to: unwanted touching, sexual assault, sexual harassment, coerced nudity, and any sexual contact with a person who is unable to provide consent due to cognitive impairment, unconsciousness, or medication effects.

Abuse (Emotional / Psychological): The intentional infliction of anguish, fear, or distress through verbal or nonverbal conduct, including but not limited to: threats, intimidation, humiliation, harassment, ridicule, isolation, silent treatment, withholding of care as punishment, and controlling behavior.

Abuse (Verbal): The use of oral, written, or gestured language that includes disparaging, derogatory, threatening, or demeaning terms, used to patients or within their hearing, regardless of their age, ability to comprehend, or disability.

Neglect: The failure of a caregiver or agency to provide the goods or services necessary to maintain the health and safety of the patient, to avoid physical harm or mental suffering, or to protect the patient from health and safety hazards. Includes:

Failure to assist with personal hygiene or nutrition

- Failure to provide necessary medical care
- Failure to prevent hazardous conditions

Failure to exercise the degree of care that a reasonable person in a like position would exercise Self-neglect (when the agency becomes aware that a patient is unable to meet their own basic needs) Exploitation (Financial): The wrongful or unauthorized taking, withholding, appropriation, or use of a patient's money, real property, personal property, or resources by any person, including:

Theft of cash, checks, credit/debit cards, or personal belongings

- Unauthorized use of a patient's financial accounts
- Coercion or undue influence to obtain control of financial assets
- Forgery of signatures on financial documents
- Identity theft

Misappropriation of Property: The deliberate misplacement, exploitation, or wrongful use of a patient's belongings or personal property, whether temporary or permanent.

Abandonment: The desertion or willful forsaking of a patient by a caregiver who has assumed responsibility for providing care, under circumstances in which a reasonable person would continue to provide care.

C. RECOGNIZING THE SIGNS

Patients, families, and visitors should be aware of potential signs of abuse, neglect, or exploitation:

Physical Indicators:

Unexplained bruises, welts, burns, fractures, or injuries

- Injuries in various stages of healing
- Injuries inconsistent with the explanation given

Signs of restraint use (wrist or ankle marks)

- Poor hygiene, unattended medical needs, pressure injuries
- Dehydration, malnutrition, or unexplained weight loss

Behavioral Indicators:

Withdrawal, fearfulness, or agitation (especially around certain individuals)

- Reluctance to speak or answer questions in the presence of a caregiver
- Changes in personality, mood, or behavior
- Depression, helplessness, or hopelessness
- Reluctance to report problems or make complaints

Financial Indicators:

Unexplained withdrawals or transfers from bank accounts

- Missing personal belongings, cash, or valuables

Sudden changes in financial documents (wills, powers of attorney)

- Unpaid bills despite adequate financial resources
- Unfamiliar signatures on financial documents
- New "best friends" or associates who isolate the patient from family

D. HOW TO REPORT

If you believe you or another patient is being abused, neglected, or exploited — whether by a caregiver, family member, friend, or agency staff member — you may report to any of the following:

AUTHORITY	CONTACT	AVAILABLE
Care Indeed Home Health	(415) 813-6888 (ask for Administrator, DON, or Compliance Officer)	24/7
Adult Protective Services (APS)	County APS: _____	24 hours
Long-Term Care Ombudsman	1-800-231-4024	Business hours
CDPH — Licensing & Certification	1-800-236-9747	Business hours
California Attorney General — Elder Abuse	(800) 952-5225	Business hours
Local Law Enforcement	_____ (non-emergency)	24 hours
911	For any situation involving immediate danger	24 hours
Medicare Fraud & Abuse	1-800-HHS-TIPS (1-800-447-8477)	Business hours

AUTHORITY	CONTACT	AVAILABLE
CMS Quality Improvement Organization (QIO)	Livanta: 1-877-588-1123	Business hours

You do NOT need proof to make a report. If you suspect abuse, neglect, or exploitation, you should report your concern. Trained investigators will determine whether abuse has occurred.

E. MANDATORY REPORTING

Under California law (Welfare & Institutions Code §15630), the following individuals are mandated reporters of suspected elder/dependent adult abuse:

All health practitioners (RNs, LVNs, PTs, OTs, SLPs, MDs, pharmacists)

- All employees of health facilities and home health agencies
- Social workers

Any person who has assumed full or intermittent responsibility for the care of an elder or dependent adult Mandated reporters must report suspected abuse to APS and/or local law enforcement within two working hours by telephone, followed by a written report (SOC 341) within the same two working days.

Failure to report is a misdemeanor punishable by up to six months in jail and/or a fine of up to \$1,000 (W&I Code §15630(h)).

F. NON-RETALIATION / WHISTLEBLOWER PROTECTION

You will NOT be retaliated against for making a good-faith report of suspected abuse, neglect, or exploitation. Your care will not be affected in any way.

Agency staff who report suspected abuse are protected from retaliation under California Labor Code §1102.5 and the federal False Claims Act whistleblower provisions (31 USC §3730(h)).

Reporters' identities are kept confidential to the extent permitted by law.

G. ACKNOWLEDGMENT

- ☐ I have been informed of my right to be free from abuse, neglect, exploitation, and misappropriation of property.
- ☐ I have been informed of how to recognize the signs of abuse, neglect, and exploitation.
- ☐ I have been provided with contact information for reporting suspected abuse, neglect, or exploitation.
- ☐ I understand that filing a report will not result in retaliation or any negative impact on my care.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Grievance & Complaint Procedure

REGULATORY BASIS 42 CFR §484.50(a)(1)(vii)–(viii); 42 CFR §484.50(c); California Health & Safety Code §1599.1(h); Title 22 CCR §74743; ACHC accreditation requirements

A. YOUR RIGHT TO FILE A GRIEVANCE

Care Indeed Home Health is committed to providing the highest quality of care. You have the absolute right to voice grievances, complaints, or suggestions about the care you receive, the services provided, the agency's policies, or the behavior of any staff member — without fear of reprisal, discrimination, penalty, or interruption of services.

A grievance is any expression of dissatisfaction with any aspect of the agency's services or operations, regardless of whether the complaint is made orally or in writing, and regardless of whether a formal resolution is sought.

B. HOW TO FILE A GRIEVANCE

Step 1 — Direct Resolution (Informal) We encourage you to raise concerns directly with your primary clinician, supervisor, or any member of your care team as soon as they arise. Many issues can be resolved quickly through direct communication.

CONTACT YOUR CARE TEAM AT
(415) 813-6888

Step 2 — Formal Written Grievance If your concern is not resolved to your satisfaction through direct communication, or if you prefer to submit a formal complaint, you may do so in writing by any of the following methods:

METHOD	DETAILS
Mail	Care Indeed Home Health, Attn: Administrator / Grievance Committee, _____ (Address)
Phone	_____
Fax	_____
Email	_____
In Person	Deliver to any staff member, who will forward it to the Administrator

A grievance form is available upon request. You may also submit a complaint in your own words — a specific form is not required.

Your grievance should include:

Your name and contact information (or you may submit anonymously; however, anonymous submissions may limit our ability to investigate and respond). A description of the concern, including dates, times, names, and locations if known. The resolution you are seeking, if any.

• Step 3 — Acknowledgment & Investigation

Acknowledgment. We will acknowledge receipt of your written grievance within five (5) business days, either in writing, by phone, or in person, as you prefer.

Investigation. The Administrator or designated Grievance Committee member will investigate the complaint. The investigation may include:

Review of relevant clinical records and documentation. Interviews with involved staff, the patient, and any witnesses. Review of agency policies and procedures. Consultation with the Director of Nursing, Compliance Officer, or legal counsel as appropriate.

Resolution. We will provide a written resolution within thirty (30) calendar days of receiving your grievance. The written resolution will include:

A summary of the investigation findings. The actions taken (or to be taken) to address the concern. Any changes to agency policies or procedures resulting from the investigation. Information about your right to escalate the complaint if not satisfied.

Extension. If the investigation requires more than 30 days (e.g., due to complexity, need for external consultation, or staff unavailability), we will notify you in writing of the expected completion date.

Step 4 — Escalation to External Agencies If you are not satisfied with the agency's resolution, or if you prefer to bypass the internal process at any time, you may file a complaint directly with the following external agencies:

AGENCY	PURPOSE	CONTACT
California Department of Public Health (CDPH)	State licensing authority for home health agencies; investigates complaints about quality of care, patient rights violations, and regulatory compliance	Phone: 1-800-236-9747 Website: www.cdph.ca.gov Mail: CDPH, L&C Program, P.O. Box 997377, MS 3000, Sacramento, CA 95899
Accreditation Commission for Health Care (ACHC)	Care Indeed's accrediting body; investigates complaints related to compliance with accreditation standards	Phone: 1-855-937-2242 Email: complaints@achc.org Website: www.achc.org
Medicare Beneficiary Ombudsman	Assists Medicare beneficiaries with complaints, grievances, and information requests	Phone: 1-800-MEDICARE (1-800-633-4227) TTY: 1-877-486-2048 Website: www.medicare.gov
Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO)	Investigates quality of care complaints for Medicare patients; handles immediate advocacy for discharge disputes	Livanta (BFCC-QIO for California): 1-877-588-1123 TTY: 1-855-887-6668
HHS Office for Civil Rights (OCR)	Investigates HIPAA privacy complaints and discrimination complaints	Phone: 1-800-368-1019 Website: www.hhs.gov/ocr/complaints
California Attorney General	Consumer protection complaints, privacy violations, elder abuse	Phone: (800) 952-5225 Website: oag.ca.gov

C. NON-RETALIATION POLICY

Care Indeed Home Health strictly prohibits retaliation against any patient, family member, or representative who files a grievance, complaint, or report with the agency or any external body. Specifically:

Your care will not be reduced, modified, or terminated because you filed a complaint. You will not be treated differently by staff as a result of your complaint. You will not be subjected to any adverse action. Any staff member who retaliates against a patient for filing a complaint will be subject to disciplinary action up to and including termination.

D. QUALITY IMPROVEMENT

All grievances, regardless of resolution, are tracked and analyzed as part of our Quality Assessment and Performance Improvement (QAPI) program. Trends in grievances are reviewed quarterly by the QAPI Committee and used to improve agency operations, policies, training, and patient outcomes.

E. ACKNOWLEDGMENT

☐ I have been informed of Care Indeed Home Health's grievance and complaint procedure.

☐ I have been provided with contact information for internal grievance filing and for external agencies.

☐ I understand that I may file a grievance at any time without fear of retaliation.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

OASIS / Assessment Consent & Data Privacy Notice

REGULATORY BASIS 42 CFR §484.55 (Comprehensive Assessment); 42 CFR §484.250 (OASIS Data); 42 CFR §484.45 (Reporting OASIS Data); CMS OASIS Guidance Manual; Privacy Act of 1974

Current instrument note: use the current CMS OASIS instrument at the time of assessment. As of this packet shell, OASIS-E2 is the active 2026 reference.

A. WHAT IS OASIS?

The Outcome and Assessment Information Set (OASIS) is the standardized CMS home health assessment instrument integrated into the comprehensive patient assessment. This packet references the current CMS OASIS instrument, including OASIS-E2 effective April 1, 2026, or a successor CMS version when released.

Demographics — Age, gender, race/ethnicity, living situation

- Medical history — Diagnoses, surgeries, overall health status

Clinical status — Vital signs, wound status, pain level, sensory status, integumentary status, respiratory status, cardiac status, neurological/emotional/behavioral status, elimination status Functional status — Ability to perform activities of daily living (ADLs) such as bathing, dressing, toileting, transferring, eating; ability to perform instrumental activities of daily living (IADLs) such as meal preparation, medication management, phone use Service utilization — Types and frequency of home health services received, hospitalizations, emergency department visits

- Medications — Drug regimen review, high-risk medications, medication management

Care management — Therapy need and plan of care, patient/caregiver involvement, discharge planning

B. WHY IS OASIS DATA COLLECTED?

OASIS data serves multiple important purposes:

Individualized Care Planning. OASIS assessment findings are used directly in developing your individualized plan of care, setting measurable goals, and identifying your specific care needs.

Quality Measurement. OASIS data is used to calculate quality measures that reflect the quality of care provided by home health agencies, including rates of improvement in mobility, self-care, and pain management; rates of hospitalization and emergency department use; and patient experience.

Public Reporting. Selected quality measures derived from OASIS data are publicly reported on CMS's Home Health Compare website ([medicare.gov/care-compare](https://www.medicare.gov/care-compare)), allowing consumers to compare the quality of home health agencies.

Medicare Reimbursement. Under the Patient-Driven Groupings Model (PDGM), OASIS data is used to determine the Medicare payment rate for each 30-day period of care, based on clinical characteristics, functional impairment level, and comorbidity.

Outcomes-Based Quality Improvement (OBQI/OBQM). Agencies use OASIS-derived outcome reports to identify areas for performance improvement through their QAPI programs.

Survey & Certification. OASIS data is used by CMS and state survey agencies in monitoring agency compliance with Medicare Conditions of Participation.

C. WHEN IS OASIS DATA COLLECTED?

OASIS assessments are performed by a qualified clinician (RN, PT, OT, or SLP) at the following time points:

TIME POINT	DESCRIPTION
Start of Care (SOC)	At the beginning of your home health episode
Resumption of Care (ROC)	When you return to home health after an inpatient stay
Recertification	At least every 60 days when your physician renews your home health certification
Follow-Up	Other time points as defined by CMS
Transfer to Inpatient Facility	When you are transferred to a hospital or other inpatient facility
Discharge	When your home health services end
Death at Home	If death occurs while under home health care

D. HOW IS OASIS DATA TRANSMITTED AND PROTECTED?

Electronic Transmission. OASIS data is transmitted electronically from Care Indeed Home Health to the CMS OASIS data repository via the Quality Improvement Evaluation System (QIES) Assessment Submission and Processing (ASAP) system, using encrypted, secure data transmission protocols.

HIPAA Protections. All OASIS data is protected health information (PHI) under HIPAA and is subject to the full range of privacy and security protections described in our Notice of Privacy Practices (§3).

CMIA Protections. OASIS data containing medical information is additionally protected under the California Confidentiality of Medical Information Act.

Privacy Act of 1974. OASIS data submitted to CMS is maintained in a system of records subject to the federal Privacy Act of 1974, which restricts how the federal government may use and disclose the data.

Access Controls. Within Care Indeed Home Health, access to OASIS data is limited to authorized clinical, quality, and billing personnel on a need-to-know basis. All access is logged and auditable.

De-identification. When OASIS data is used for public reporting or research, it is de-identified in accordance with HIPAA standards to protect your identity.

E. YOUR RIGHTS REGARDING OASIS DATA

Right to Review. You have the right to review the OASIS data collected about you. To request a review, contact our Privacy Officer or Director of Nursing.

Right to Request Corrections. If you believe that OASIS data collected about you is inaccurate, you have the right to request a correction. Your request will be reviewed by the assessing clinician and the Director of Nursing. If the request is substantiated, the OASIS assessment will be corrected and retransmitted to CMS in accordance with CMS correction policies.

Right to Accounting. You may request an accounting of disclosures of your OASIS data as described in the Notice of Privacy Practices (§3).

Right to Restrict. You may request restrictions on the use or disclosure of your OASIS data beyond what is required by law. However, because OASIS collection and transmission is mandated by CMS as a condition of Medicare certification, the agency is required to collect and transmit OASIS data for all eligible patients and cannot honor a request to restrict the CMS-mandated collection and transmission.

F. CONSENT AND ACKNOWLEDGMENT

By signing below, I acknowledge that:

I have been informed about the OASIS assessment process, including what data is collected, when it is collected, how it is used, and how it is protected.

I understand that OASIS data will be collected, used, and transmitted as required by applicable CMS home health assessment and quality-reporting rules, including OASIS-E2 or successor CMS instrument requirements.

I understand that my OASIS-derived quality measures may be publicly reported on the CMS Home Health Compare website in a de-identified, aggregate format.

I have been informed of my right to review and request corrections to my OASIS data.

I consent to the comprehensive assessment process, including the collection, use, and electronic transmission of OASIS data.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Homebound Status Acknowledgment

REGULATORY BASIS 42 CFR §484.55(a)(1); Social Security Act §1814(a)(2)(C) and §1835(a)(2)(A); Medicare Benefit Policy Manual Chapter 7, §30.1; CMS Transmittal Updates

A. WHAT IS HOMEBOUND STATUS?

To be eligible for Medicare-covered home health services, you must meet the Medicare homebound (confined to the home) criteria. You are considered homebound if:

Criterion 1 — There exists a normal inability to leave the home, AND leaving home requires a considerable and taxing effort.

This may be due to:

A condition that restricts your ability to ambulate (paralysis, amputation, severe weakness, orthopedic impairment, stroke effects, severe shortness of breath, etc.) Need for assistive devices (walker, wheelchair, crutches, cane) or the assistance of another person to leave home A medical condition that makes leaving home medically contraindicated (e.g., immunosuppression, unstable cardiac condition, risk of falls) Cognitive or psychiatric impairment that renders you unable to safely leave home unassisted (e.g., dementia, severe anxiety, agoraphobia)

- Dependence on life-sustaining technology that is not easily portable

And

Criterion 2 — Absences from the home are infrequent, of short duration, or to receive medical treatment.

The following activities generally do not disqualify you from homebound status:

Attendance at adult day care programs (licensed/certified/accredited/state-regulated)

- Attendance at religious services

Trips for medical treatment (doctor's appointments, dialysis, chemotherapy, radiation, etc.) Short, infrequent walks or trips outside (e.g., trip to the barber, brief walk around the block)

- Attendance at funerals, graduations, or other unique events

Important: Homebound status is a clinical determination made by your care team and physician based on your individual circumstances. Meeting the homebound criteria requires both Criterion 1 AND Criterion 2.

B. HOW HOMEBOUND STATUS IS ASSESSED AND DOCUMENTED

Initial Assessment. Your homebound status will be assessed during your initial comprehensive assessment (OASIS Start of Care) and documented in your clinical record.

Ongoing Assessment. Your homebound status will be reassessed at each skilled visit and documented in the visit note.

Clinicians will assess and record specific clinical findings that support homebound status, including:

Nature of the condition that restricts mobility or the ability to leave home Specific functional limitations (e.g., ambulates with rolling walker, maximum distance 30 feet with fatigue) Taxing effort required to leave home (e.g., needs assistance of one person to transfer from wheelchair to car; develops severe dyspnea with minimal exertion)

- Frequency and purpose of any absences from home

Recertification. Homebound status is recertified by your physician at each 60-day recertification period. Your physician must certify that you meet the homebound criteria as a condition of continued Medicare coverage.

Changes in Status. If your functional status improves to the point that you no longer meet the homebound criteria, your care team will:

Notify you and discuss the implications for your Medicare home health benefit. Communicate with your physician regarding your changed status. Provide you with written notice before any change in service. Assist you in identifying alternative resources or settings for continued care.

C. YOUR RESPONSIBILITIES

Be Honest. Accurately report your functional abilities, limitations, and activities to your care team. Do not overstate or understate your abilities.

Report Changes. Promptly inform Care Indeed Home Health if your condition significantly improves or declines, or if your activities outside the home change substantially.

Understand the Impact. You understand that if you do not meet the homebound criteria, your Medicare home health benefit may be affected, including potential denial of claims for services rendered. This could result in:

Medicare denying coverage for home health services The issuance of an Advance Beneficiary Notice (ABN)

- Potential financial responsibility for non-covered services

Cooperate with Assessment. Allow your clinicians to accurately assess your functional status, mobility, and ability to leave home.

D. ACKNOWLEDGMENT

- ☐ I have been informed of the Medicare homebound requirement and its criteria.
- ☐ I understand that homebound status is assessed at each visit and recertified every 60 days.
- ☐ I understand that certain absences from home (medical appointments, religious services, adult day care, etc.) generally do not disqualify me from homebound status.
- ☐ I understand that if I no longer meet homebound criteria, my Medicare home health benefit may be affected, and I will be notified before any change in service.
- ☐ I agree to honestly report my functional abilities and activities to my care team.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Home Safety Assessment Consent

REGULATORY BASIS 42 CFR §484.55(b)(5)–(6) (Comprehensive Assessment — patient home environment); 42 CFR §484.60(a) (Plan of Care — safety measures); Title 22 CCR §74663; ACHC accreditation requirements; Cal/OSHA Title 8

A. PURPOSE

A comprehensive home safety assessment is a required component of your initial assessment and is essential to ensuring the safety of both you and your care team. A registered nurse or therapist will evaluate your home environment to identify and address potential hazards that could affect your health, safety, and the safe delivery of care.

B. SCOPE OF ASSESSMENT

The home safety assessment includes evaluation of the following areas:

1. Fall Prevention

Flooring surfaces (loose rugs, uneven surfaces, wet floors) Lighting adequacy (entrances, hallways, stairwells, bathroom, bedroom) Stairway safety (handrails, condition, lighting) Bathroom safety (grab bars, non-slip mats, shower bench/chair, elevated toilet seat) Bedroom safety (bed height, path from bed to bathroom, nightlight)

- Clutter, cords, and obstacles in walkways
- Footwear evaluation

Assistive device condition (walker, cane, wheelchair — proper fit, working brakes, worn tips)

2. Fire and Electrical Safety

Smoke detectors (presence, function, battery status — at least one on each level) Carbon monoxide detectors (presence and function) Fire extinguisher (presence, access, expiration) Emergency exit routes (accessible, unobstructed, known to patient) Electrical hazards (frayed cords, overloaded outlets, space heater safety)

- Candle, open flame, and smoking safety
- Oxygen safety precautions (if applicable — no open flame within 10 feet, proper storage, signage)

3. Medication Safety

Medication storage (secure, organized, appropriate temperature, away from children)

- Expiration date review
- Medication reconciliation with physician orders
- Controlled substance storage and security

Medication administration aids (pill organizer, medication reminder devices) Sharps disposal (if applicable — proper sharps container)

4. Infection Control

Handwashing facilities (running water, soap, clean towels accessible)

- Bathroom sanitation
- Kitchen sanitation

Waste disposal (general and biohazardous, if applicable) Pest control (evidence of rodents, insects, or infestations)

- Availability of personal protective equipment (PPE) supplies

5. Equipment and Supply Safety

Durable medical equipment condition and function (hospital bed, oxygen concentrator, nebulizer, wound VAC, IV pump, etc.)

- Proper use and maintenance of equipment
- Electrical requirements for medical equipment
- Backup power source for life-sustaining equipment

Supply storage (clean, dry, accessible)

6. Environmental Hazards

Structural integrity (walls, floors, ceilings, roof leaks) Temperature extremes (adequate heating, cooling, insulation) Water safety (hot water temperature — should not exceed 120°F) Lead paint (older homes with young children or pregnant caregivers) Asbestos concerns (older homes undergoing renovation)

- Pest/rodent infestation
- Mold or mildew
- Chemical/hazardous material storage

7. Security and Emergency Access

Door locks, deadbolts, and access for emergency personnel

- Key/lockbox availability for caregivers and emergency responders

Address visibility from street (for EMS response) Communication devices (phone access, medical alert system)

- Emergency numbers posted in visible location

Home alarm system (access codes for caregivers if needed) Weapon storage (firearms, knives, etc. — properly secured and stored)

8. Personal Safety

Presence of aggressive or unrestrained animals

- Presence of individuals who may pose a threat to patient or staff
- Evidence of substance use or illegal activity
- Evidence of domestic violence or unsafe relationships
- Neighborhood safety concerns affecting care delivery

C. CONSENT AND UNDERSTANDING

By signing below, I consent to the home safety assessment and acknowledge the following:

A qualified clinician will conduct a thorough assessment of my home environment during the initial visit and periodically thereafter.

Recommendations to address identified hazards will be discussed with me, documented in my clinical record, and incorporated into my plan of care.

I am not obligated to follow all recommendations. However, if I choose not to implement a recommendation, my decision and the potential consequences will be documented in my medical record.

If conditions in the home pose an imminent threat to the safety of the patient or agency staff (e.g., structural collapse risk, gas leak, violent individual present, active hazardous material exposure), Care Indeed Home Health may:

Temporarily suspend visits until the hazard is remediated. Modify the plan of care to address safety concerns. Notify emergency services (911) if an immediate life-safety threat exists. In extreme cases, discharge from services if hazards are persistent, unmitigable, and pose an unacceptable safety risk — with appropriate notice and referral. Safety assessments are an ongoing process. Your care team will reassess safety at each visit and formally at each recertification.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Plan Of Care Acknowledgment

REGULATORY BASIS 42 CFR §484.60; 42 CFR §484.55; Medicare Benefit Policy Manual Ch. 7, §30; Title 22 CCR §74661; ACHC accreditation requirements

A. WHAT IS THE PLAN OF CARE?

Your plan of care (POC), also known as the CMS-485 or home health certification and plan of care, is a comprehensive written document that directs your home health services. It is:

Developed by your attending physician in collaboration with your Care Indeed Home Health clinical team and with your active participation. Individualized to your specific diagnoses, needs, goals, and preferences. Signed by your attending physician, certifying the medical necessity of home health services.

B. WHAT DOES THE PLAN OF CARE INCLUDE?

Your plan of care includes, at a minimum:

Diagnoses — All pertinent diagnoses, including the primary diagnosis that is the principal reason for home health services. Mental/cognitive and functional status — Assessment of your cognitive, behavioral, and functional abilities and limitations. Types of services — The specific disciplines providing care (nursing, PT, OT, SLP, MSW, HHA). Frequency and duration of visits — How often each discipline will visit and for how long (e.g., "SN 3x/week x 2 weeks, then 2x/week x 2 weeks"). Goals and outcomes — Measurable, time-bound goals for improvement or maintenance of your health and function.

Medications — Complete medication list including dose, frequency, route, and purpose. Treatments and procedures — Wound care protocols, IV therapy, exercises, teaching plans, etc. Diet — Any dietary restrictions or nutritional interventions. Activity orders — Any restrictions or recommendations regarding physical activity and mobility. DME and supplies — Durable medical equipment and supplies needed. Safety measures — Interventions to address fall risk, medication safety, emergency preparedness, and other safety concerns identified in the home safety assessment.

Discharge plan — Anticipated discharge criteria, patient/caregiver education plan, and community resources. Patient/caregiver education — Specific teaching topics and learning goals. Advance directive status — Documentation of whether advance directives are in place and their contents.

C. REVIEW, UPDATES, AND RECERTIFICATION

Initial Certification. Your physician must sign and date the initial plan of care before billing can occur. Care Indeed Home Health will obtain the signed POC and maintain it in your clinical record.

60-Day Recertification. Your plan of care is reviewed and recertified by your physician at least every 60 days (each recertification period). The recertification includes:

Reassessment of homebound status

- Review of all active diagnoses, medications, and orders
- Evaluation of progress toward goals
- Adjustment of services as indicated
- Physician signature on the updated plan

Significant Change in Condition. If you experience a significant change in health status (e.g., hospitalization, new diagnosis, major functional decline or improvement, fall, medication change), your plan of care will be updated promptly to reflect the change. A comprehensive reassessment (OASIS) may be performed.

Verbal Orders. When immediate changes to your care are needed, your clinician may obtain verbal orders from your physician. These orders are documented, implemented immediately, and sent to your physician for signature within the timeframe required.

by regulation.

Care Conferences. Your care team conducts regular case conferences to coordinate care, review progress, discuss concerns, and update the plan of care. You and/or your representative are welcome to participate in care conferences upon request.

D. YOUR RIGHTS AND PARTICIPATION

You have the right to participate in the development and revision of your plan of care. Your preferences, goals, and values will be considered.

You have the right to be informed of your plan of care, including all services ordered, their frequency, and your goals. You will receive a written copy upon request.

You have the right to be notified of any changes to your plan of care before the change is made (except in emergencies where delay could jeopardize your health or safety).

You have the right to refuse any service or treatment included in your plan of care, after being informed of the potential consequences.

You have the right to request changes to your plan of care, which will be communicated to your physician for consideration.

E. ACKNOWLEDGMENT

- ☐ I have been informed about my plan of care and understand its purpose.
- ☐ I understand that my plan of care is developed by my physician in collaboration with the care team and with my participation.
- ☐ I understand that my plan of care is reviewed and recertified at least every 60 days.
- ☐ I understand that I will be notified of any changes to my plan of care before the change is made.
- ☐ I understand that I have the right to participate in my plan of care, to request changes, and to refuse any service.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Physician Certification & Face-To-Face Encounter Acknowledgment

REGULATORY BASIS 42 CFR §484.55(a); 42 CFR §424.22(a)–(d); Social Security Act §1814(a)(2)(C), §1835(a)(2)(A); Medicare Benefit Policy Manual Ch. 7, §30.5; CMS Face-to-Face Encounter Guidance

A. PHYSICIAN CERTIFICATION REQUIREMENT

For your home health services to be covered by Medicare, your attending physician (or an allowed non-physician practitioner under certain circumstances) must certify that:

You are homebound — You meet the Medicare homebound criteria as described in §13. You need skilled services — You require intermittent skilled nursing care, physical therapy, speech-language pathology services, or have a continuing need for occupational therapy. A plan of care has been established — An individualized plan of care has been established and is periodically reviewed by the physician. You are under the care of a physician — The services are or were provided while you are under the care of a physician who is qualified to practice under Medicare.

B. FACE-TO-FACE ENCOUNTER REQUIREMENT

In addition to the certification, Medicare requires that a face-to-face encounter occur between you and your certifying physician (or an allowed non-physician practitioner — nurse practitioner, clinical nurse specialist, physician assistant, or certified nurse midwife — working in collaboration with or under the supervision of the certifying physician).

Timing: The face-to-face encounter must occur within the 90 days before or the 30 days after the start of your home health episode of care.

Purpose: The encounter must be related to the primary reason you need home health services. The practitioner who performs the encounter must document the clinical findings that support your homebound status and need for skilled services.

Documentation: The certifying physician must document that the face-to-face encounter occurred, the date of the encounter, and the encounter findings that support homebound status and the need for skilled services. This documentation must be a separate and distinct section of the certification.

Telehealth: CMS permits the face-to-face encounter to be conducted via telehealth in accordance with applicable Medicare telehealth policies.

C. RECERTIFICATION

Your physician must recertify your need for home health services at the beginning of each 60-day recertification period for your care to continue under Medicare.

Recertification requires the physician to certify that you continue to meet the homebound criteria and continue to need skilled services.

A new face-to-face encounter is not required at each recertification. However, your physician must have sufficient knowledge of your current clinical condition through direct care, consultation with the home health agency, or review of clinical documentation.

D. YOUR RESPONSIBILITY

You understand that your home health services are provided under physician orders and that a valid physician certification is a condition of Medicare coverage.

You understand that if your physician does not sign or renew the certification in a timely manner, it may result in a delay or interruption of services until the certification is obtained.

You agree to keep scheduled appointments with your physician or to communicate with your physician as requested to support the certification and plan of care process.

E. ACKNOWLEDGMENT

- ☐ I understand that my home health services require a physician's order, certification, and periodic recertification (at least every 60 days).
- ☐ I understand that a face-to-face encounter with my physician (or allowed NPP) is required within the timeframes specified by Medicare.
- ☐ I understand that if certification or recertification is not obtained, my Medicare home health services may be affected.
- ☐ I agree to cooperate with the physician certification and face-to-face encounter process.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Infection Prevention & Control Program Summary

REGULATORY BASIS 42 CFR §484.70; Cal/OSHA Title 8 §5193 (Bloodborne Pathogens) and §5199 (Aerosol Transmissible Diseases); California Health & Safety Code §1250.6, §120175 et seq.; ACHC accreditation requirements

A. PROGRAM OVERVIEW

Care Indeed Home Health maintains a comprehensive Infection Prevention and Control (IPC) Program designed to prevent and control the transmission of infections among patients, staff, caregivers, and visitors. The program is supervised by a qualified professional and is integrated into our QAPI program.

Our IPC Program includes:

Written infection control policies and procedures reviewed at least annually

- Ongoing surveillance of infections and communicable diseases
- Staff education and competency evaluation in infection prevention

Compliance with OSHA/Cal-OSHA bloodborne pathogen and aerosol transmissible disease standards

- Antibiotic stewardship activities
- Outbreak investigation and response protocols
- Coordination with local and state public health authorities

B. STANDARD PRECAUTIONS

Standard Precautions are the minimum infection prevention practices that apply to all patient care, regardless of suspected or confirmed infection status. All Care Indeed Home Health staff are trained in and will practice the following:

1. Hand Hygiene

Staff will perform hand hygiene (handwashing with soap and water or use of alcohol-based hand sanitizer containing at least 60% alcohol) before and after every patient contact, after removing gloves, after contact with body fluids or contaminated surfaces, and before any clean/aseptic procedure.

Hand hygiene is the single most important measure to prevent the spread of infection.

2. Personal Protective Equipment (PPE)

Gloves — Worn when contact with blood, body fluids, mucous membranes, non-intact skin, or contaminated items is anticipated. Changed between tasks and between patients. Masks and Respiratory Protection — Worn when there is risk of splash/spray to the face, when caring for patients with respiratory infections (N95 or higher for aerosol transmissible diseases as indicated), and during procedures that generate aerosols. Gowns — Worn when contact with blood, body fluids, or contaminated surfaces is anticipated, and when the staff member's clothing may become contaminated.

Eye Protection (goggles/face shield) — Worn when splash or spray of blood or body fluids to the eyes is anticipated.

3. Sharps Safety

Needles, lancets, and other sharps are never recapped, bent, or broken by hand. All sharps are disposed of immediately after use into an approved, puncture-resistant sharps container. Sharps containers in the patient's home are provided by the agency and are disposed of according to local regulations.

4. Safe Injection Practices

Single-use vials are used for one patient and discarded after use. Multi-dose vials, if used, are dedicated to a single patient whenever possible and stored per manufacturer's instructions. Injection equipment (syringes, needles, prefilled devices) is never used on more than one patient.

5. Respiratory Hygiene / Cough Etiquette

Patients and household members with respiratory symptoms are encouraged to cover coughs and sneezes, use tissues (disposed immediately), and perform hand hygiene.

Staff will don a mask when caring for patients with respiratory symptoms, and provide the patient with a mask if appropriate.

6. Environmental Cleaning

Reusable patient care equipment is cleaned and disinfected between patients using EPA-registered hospital-grade disinfectants. Single-use items are disposed of after one use.

Staff will clean and disinfect the work area and equipment at each visit as needed.

7. Linen and Laundry

Contaminated linens are handled with minimum agitation to avoid aerosolization. Patient/family is instructed on proper handling of soiled linens (washing in hot water with detergent).

8. Waste Management

Regulated medical waste (blood-soaked materials, sharps, etc.) is segregated, contained, and disposed of in accordance with California Medical Waste Management Act (Health & Safety Code §117600 et seq.) and local regulations. General waste is disposed of according to standard household practices.

C. TRANSMISSION-BASED PRECAUTIONS

When a patient is known or suspected to have an infection that requires precautions beyond Standard Precautions, the following additional measures may be implemented:

Type	When Used	Additional Precautions
Contact Precautions	MRSA, VRE, C. diff, scabies, impetigo, wound infections with resistant organisms	Gown and gloves for all patient contact; dedicated equipment; enhanced environmental cleaning
Droplet Precautions	Influenza, pertussis, meningococcal disease, respiratory viruses	Surgical mask within 6 feet of patient; patient wears mask during close contact
Airborne Precautions	Tuberculosis (active pulmonary), measles, varicella, COVID-19 (per current guidance)	N95 respirator (fit-tested); limit exposure time; maximize ventilation; coordinate with public health

Patients and household members will be informed of any transmission-based precautions and educated on their role in preventing spread.

D. ANTIBIOTIC STEWARDSHIP

Care Indeed Home Health supports antibiotic stewardship principles:

Antibiotics are administered only as prescribed by the patient's physician. Clinicians will monitor for signs of antibiotic-related adverse effects, including *Clostridioides difficile* (C. diff) infection. Patient/caregiver education includes the importance of completing prescribed antibiotic courses, not sharing antibiotics, and recognizing signs of drug-resistant infections. Antibiotic-related concerns are communicated to the prescribing physician for evaluation.

E. COMMUNICABLE DISEASE REPORTING

Care Indeed Home Health complies with all California mandatory communicable disease reporting requirements under Health & Safety Code §120175 et seq. and Title 17 CCR §2500. If a reportable communicable disease is identified or suspected, the appropriate local health department and/or CDPH will be notified within the required timeframes.

F. PATIENT AND CAREGIVER ROLE IN INFECTION PREVENTION

You and your caregivers play a critical role in preventing infections:

Inform your care team immediately if you develop any new signs or symptoms of infection:

Fever (temperature $\geq 100.4^{\circ}\text{F}$ / 38°C) or chills

- New or worsening cough, shortness of breath, or sore throat
- Rash, skin redness, or open sores

Diarrhea (especially if taking or recently completed antibiotics) Wound drainage (increased amount, change in color/odor)

Urinary symptoms (burning, frequency, urgency, cloudy/foul-smelling urine)

- Redness, swelling, or drainage at IV or catheter sites

Maintain a clean care area. Keep surfaces in the care area clean. Regularly clean high-touch surfaces (doorknobs, light switches, bed rails, bathroom fixtures).

Provide handwashing facilities. Ensure running water, liquid soap, and clean towels (paper towels preferred) are available near the care area for staff hand hygiene. If running water is unavailable, provide alcohol-based hand sanitizer.

Proper sharps disposal. If you have sharps containers in the home, do not overfill them past the fill line. Contact the agency for replacement containers.

Follow isolation instructions. If transmission-based precautions are in place, follow all instructions provided by your clinician.

Notify the agency if any household member develops symptoms of a communicable disease (flu, COVID-19, TB, gastroenteritis, etc.) so appropriate precautions can be taken during visits.

Practice respiratory hygiene. Cover coughs and sneezes, dispose of tissues immediately, and perform hand hygiene afterward.

G. ACKNOWLEDGMENT

- ☐ I have received and understand the Infection Prevention & Control Program Summary.
- ☐ I understand my role and my caregiver's role in preventing infections.
- ☐ I have been informed of the signs and symptoms of infection that I should report immediately.
- ☐ I agree to maintain handwashing facilities and a clean care area for agency staff.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Cultural & Linguistic Services Notice

REGULATORY BASIS

Title VI of the Civil Rights Act of 1964; Section 1557 of the Affordable Care Act; Executive Order 13166; California Government Code §11135; California Health & Safety Code §1259; Dymally-Alatorre Bilingual Services Act (Gov. Code §7290 et seq.); 42 CFR §484.50; ACHC accreditation requirements

A. NON-DISCRIMINATION STATEMENT

Care Indeed Home Health complies with applicable federal civil rights laws and does not discriminate on the basis of race, color, national origin, age, disability, sex, sexual orientation, gender identity, religion, ancestry, marital status, genetic information, or any other characteristic protected by law. Care Indeed Home Health does not exclude people or treat them differently because of any protected characteristic.

Care Indeed Home Health:

Provides free aids and services to people with disabilities to communicate effectively with us, including qualified sign language interpreters, written information in other formats (large print, audio, accessible electronic formats, other formats). Provides free language services to people whose primary language is not English, including qualified medical interpreters and information written in other languages.

IF YOU NEED THESE SERVICES, CONTACT

B. LANGUAGE ACCESS SERVICES

The following services are available to you at no charge:

1. Interpreter Services

In-Person Interpreters — Qualified medical interpreters are available for scheduled visits and assessments. Requests should be made at least 48 hours in advance when possible. Telephone / Video Interpreters — On-demand interpreter services (language line) are available 24 hours a day, 7 days a week for immediate communication needs, including unscheduled visits and after-hours calls. Qualified Medical Interpreters — All interpreters used for clinical encounters meet the requirements for a "qualified medical interpreter" under California law and Section 1557, including proficiency in both English and the target language, knowledge of specialized medical terminology, and understanding of interpreter ethics (including confidentiality).

Family Members / Friends as Interpreters — Care Indeed Home Health will not rely on minor children or adult family members/friends to interpret during clinical encounters unless the patient specifically requests it and a qualified interpreter has first been offered. The patient's preference will be documented. Minors will not be used as interpreters except in an emergency.

2. Translated Documents

Key documents, including this Patient Admission Packet and the Notice of Privacy Practices, are available in the following languages:

_____, _____, _____, _____, _____

(Insert languages based on the top language populations in the service area, per the requirements of the Dymally-Alatorre Bilingual Services Act and Section 1557 taglines)

If your language is not listed, we will arrange for translation of essential documents upon request.

3. Alternative Formats

Documents are available in the following alternative formats upon request:

Large print (minimum 18-point font)

- Audio recording
- Braille
- Electronic format compatible with screen readers

C. CULTURAL ACCOMMODATIONS

Care Indeed Home Health recognizes that your cultural background, beliefs, and practices are important to your health and well-being. We are committed to providing culturally competent care that respects your individuality. The following accommodations are available:

Cultural Preferences in Care. Your cultural, spiritual, and dietary preferences will be assessed during the initial evaluation and incorporated into your plan of care to the extent medically appropriate and operationally feasible.

Dietary Restrictions. If you follow specific dietary practices for cultural, religious, or personal reasons (e.g., kosher, halal, vegetarian, fasting), inform your care team so that nutritional counseling and meal planning can be appropriately tailored.

Spiritual and Religious Practices. You may designate spiritual or religious practices that you wish to be observed during care delivery (e.g., prayer times, modesty requirements, specific end-of-life rituals). Your care team will accommodate these to the extent possible.

Caregiver Gender Preference. You may request a caregiver of a specific gender for personal care services (bathing, dressing, toileting). We will make every reasonable effort to accommodate your preference, subject to staff availability.

Traditional and Complementary Practices. If you use traditional, cultural, or complementary health practices (e.g., herbal remedies, traditional medicine, acupuncture), please inform your care team. We will not discourage these practices unless they conflict with your medical treatment; in that case, we will discuss the concern with you and your physician.

Family Involvement. Some cultures place a high value on family involvement in medical decision-making. We welcome family participation to the extent you desire and authorize.

Death and Dying. Cultural and religious practices surrounding death and dying will be respected. Please inform your care team of any specific wishes related to end-of-life care, handling of the body, or post-mortem practices.

D. SECTION 1557 TAGLINES

[Insert CMS-required taglines in the top 15 languages in the state, informing individuals with limited English proficiency of the availability of language assistance services. The taglines should read substantially as follows in each language:]

ATTENTION: If you speak [Language], language assistance services, free of charge, are available to you. Call _____ (TTY: _____).

(Repeat in: Spanish, Chinese, Tagalog, Vietnamese, Korean, Armenian, Farsi, Russian, Japanese, Arabic, Punjabi, Khmer, Hmong, Hindi, Thai — or as required by current HHS guidance and California data)

E. HOW TO FILE A DISCRIMINATION COMPLAINT

If you believe Care Indeed Home Health has failed to provide these services or has discriminated in any way on the basis of race, color, national origin, age, disability, or sex, you may file a grievance with:

Care Indeed Home Health — Compliance Officer Name: _____ Phone: _____ Email: _____ Address: _____

You may also file a civil rights complaint with:

U.S. Department of Health and Human Services — Office for Civil Rights Phone: 1-800-368-1019 | TDD: 1-800-537-7697
Online: www.hhs.gov/ocr/complaints Mail: 200 Independence Avenue SW, Room 509F, HHH Building, Washington, DC 20201

California Department of Fair Employment and Housing (DFEH) Phone: (800) 884-1684 | TTY: (800) 700-2320 Online: www.dfeh.ca.gov

F. ACKNOWLEDGMENT

- ☐ I have been informed of Care Indeed Home Health's non-discrimination policy.
- ☐ I have been informed of the availability of free language assistance services, including interpreters and translated documents.
- ☐ I have been informed of the availability of free aids and services for persons with disabilities.
- ☐ I have been asked about my cultural, spiritual, dietary, and language preferences, and these have been documented.
- ☐ I understand how to file a discrimination complaint with the agency and with external civil rights agencies.

PRIMARY LANGUAGE

CULTURAL / RELIGIOUS PREFERENCES TO NOTE

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Telehealth / Remote Patient Monitoring Consent

REGULATORY BASIS California Business & Professions Code §2290.5 (Telehealth); 42 CFR §484.65(d) (QAPI — use of technology); CMS Medicare Benefit Policy Manual Ch. 7 (home health telehealth provisions); California Civil Code §56.06 (electronic health records); ACHC Telehealth Standards

Conditional telehealth/RPM section: render or separately task this section only if telehealth or remote patient monitoring is offered, selected, or included in the plan of care.

This section applies ONLY if telehealth and/or remote patient monitoring (RPM) services are included in your plan of care. If these services are not ordered, this section may be skipped.

A. DEFINITIONS

Telehealth — The mode of delivering health care services using information and communication technologies to facilitate the diagnosis, consultation, treatment, education, care management, and self-management of a patient at a distance from the health care provider. In the context of home health, telehealth includes real-time audio/video visits between the patient (at home) and a clinician (at the agency or remote location).

Remote Patient Monitoring (RPM) — The use of connected digital technologies to collect physiological data (vital signs, weight, blood glucose, oxygen saturation, etc.) from the patient at home and electronically transmit that data to the care team for assessment, monitoring, and clinical decision-making.

Store-and-Forward — The transmission of recorded health information (images, data, documents) through secure electronic communication for review by a provider at a later time.

B. SERVICES THAT MAY BE PROVIDED VIA TELEHEALTH / RPM

Depending on your plan of care, the following services may be delivered or supplemented via telehealth or RPM:

Virtual Nursing Visits — Assessment, education, medication review, wound monitoring (via patient/caregiver-submitted photographs), symptom management, care coordination. **Virtual Therapy Sessions** — Home exercise program monitoring and progression, functional assessment via video observation, patient/caregiver education (PT/OT/SLP). **Vital Signs Monitoring** — Daily or scheduled transmission of:

- Blood pressure
- Heart rate / pulse

Oxygen saturation (SpO2)

- Weight
- Blood glucose
- Temperature
- Other parameters as ordered

Symptom Surveys — Scheduled electronic questionnaires about symptoms, pain, medication adherence, and functional status.

Secure Messaging — Asynchronous communication with your care team for non-urgent questions, appointment coordination, and follow-up. **Virtual Care Conferences** — Family conferences, care coordination meetings, and interdisciplinary team discussions conducted via video.

C. BENEFITS AND LIMITATIONS

Benefits:

Increased access to your care team between in-person visits. Earlier detection of changes in your condition through daily monitoring. Reduced need for some in-person visits (may reduce disruption to your routine). Enhanced patient and caregiver engagement in self-management. Continuity of care during emergencies, severe weather, or public health events. Limitations:

Telehealth is not a substitute for in-person care when a hands-on assessment, procedure, or physical intervention is required. The quality of the telehealth encounter depends on the reliability of technology (internet connection, audio/video quality, device functionality). There are inherent limitations in the ability to perform a physical examination remotely. Some conditions may not be adequately assessed via telehealth. Technology failures (internet outages, device malfunction, power loss) may interrupt telehealth services. In such cases, the agency will arrange an alternative means of communication or an in-person visit.

Privacy risks exist with any electronic communication. While we use encrypted, HIPAA-compliant platforms, no system can guarantee absolute security. See §3 (Notice of Privacy Practices) for details.

D. TECHNOLOGY REQUIREMENTS

For Video Visits:

A device with a camera and microphone (smartphone, tablet, or computer). A reliable internet connection (recommended: minimum 1.5 Mbps upload/download speed). A private, quiet location with adequate lighting. For Remote Patient Monitoring:

RPM devices will be provided by Care Indeed Home Health or a contracted vendor. Devices may include: Bluetooth blood pressure monitor, pulse oximeter, scale, glucometer, and/or thermometer. Training on the use of RPM equipment will be provided by your clinician during an in-person visit. A compatible smartphone, tablet, or hub device for data transmission (provided or specified by the agency).

E. PRIVACY AND SECURITY

All telehealth encounters are conducted using HIPAA-compliant, encrypted technology platforms. Telehealth encounters are part of your official medical record and subject to the same privacy and confidentiality protections as in-person visits (see §3). Audio/video recordings of telehealth encounters will not be made without your express written consent. RPM data is transmitted through encrypted channels and stored in our secure EHR system. You should conduct video visits from a private location in your home to protect your own privacy. The agency is not responsible for breaches of privacy caused by the patient conducting a visit in a public or shared space.

F. CONSENT AND UNDERSTANDING

By signing below, I acknowledge and agree that:

I have been informed of the nature, benefits, limitations, and privacy considerations of telehealth and/or remote patient monitoring services.

I understand that telehealth supplements — but does not replace — in-person home health services.

I understand the technology requirements and will make reasonable efforts to maintain the equipment and connectivity needed for telehealth/RPM services.

I consent to the use of the following platforms/devices: _____ (or "as determined by the care team").

I understand that I may revoke consent for telehealth/RPM services at any time, in writing, without affecting my eligibility for in-person home health services.

I understand that in an emergency, I should call 911 — telehealth is not a substitute for emergency medical services.

I understand that technology failures may occur, and the agency will arrange alternative communication or an in-person visit if needed.

I consent to the use of RPM data collected from monitoring devices as part of my clinical assessment and plan of care.

- ☐ I CONSENT to telehealth and/or remote patient monitoring services as described above and as included in my plan of care.
- ☐ I DECLINE telehealth and/or remote patient monitoring services at this time. I understand I may request them in the future.

Acknowledgment for this section is captured on the Master Signature Page unless a separate standalone form is required.

Master Signature Page & Packet Checklist

A. MASTER SIGNATURE PAGE

This master signature confirms packet receipt, review, questions answered, and consent to initiate ordered home health services. Separate eCIGN tasks/forms are generated only when HIPAA ROI, Private Pay, Telehealth/RPM, interpreter, witness, or CMS notice triggers apply.

By signing below, I confirm the following:

I have received, reviewed, and understand all sections of this Patient Admission Packet (§1 through §19).

Each section has been explained to me verbally, in a language and manner I understand, by my admitting clinician.

I have had the opportunity to ask questions about every section, and my questions have been answered to my satisfaction.

I have received copies of the following documents:

#	DOCUMENT	RECEIVED (□)
1	Welcome Letter & Agency Identification (§1)	☐
2	Patient Rights & Responsibilities (§2)	☐
3	Notice of Privacy Practices — HIPAA/CMIA (§3)	☐
4	Consent to Treat (§4)	☐
5	Authorization for Use & Disclosure of PHI — HIPAA ROI (§5)	☐
6	Advance Directive Information & Acknowledgment (§6)	☐
7	Financial Agreement & Payment Terms (§7)	☐
8	Private-Pay Service Agreement (§8) (if applicable)	☐ / N/A
9	Emergency Preparedness Plan Summary (§9)	☐
10	Abuse, Neglect & Exploitation Reporting Notice (§10)	☐
11	Grievance & Complaint Procedure (§11)	☐
12	OASIS / Assessment Consent & Data Privacy Notice (§12)	☐
13	Homebound Status Acknowledgment (§13)	☐
14	Home Safety Assessment Consent (§14)	☐
15	Plan of Care Acknowledgment (§15)	☐
16	Physician Certification & F2F Encounter Acknowledgment (§16)	☐
17	Infection Prevention & Control Program Summary (§17)	☐
18	Cultural & Linguistic Services Notice (§18)	☐
19	Telehealth / RPM Consent (§19) (if applicable)	☐ / N/A

I understand that I may request additional copies of any document at any time.

I consent to the initiation of home health services as described in this packet and as ordered by my physician.

Patient / Authorized Representative

SIGNATURE

_____ Date: _____ Time: _____

PRINT NAME

RELATIONSHIP TO PATIENT (IF REPRESENTATIVE)

LEGAL AUTHORITY (E.G., POA, CONSERVATOR, COURT ORDER)

REASON PATIENT CANNOT SIGN (IF APPLICABLE)

Admitting Clinician

SIGNATURE

_____ Date: _____ Time: _____

PRINT NAME

TITLE / CREDENTIALS

WITNESS (required when patient is unable to sign, when a representative signs, or when interpreter is used)

SIGNATURE

_____ Date: _____ Time: _____

PRINT NAME

TITLE

INTERPRETER (if used during admission)

☐ An interpreter was used during the admission process.

LANGUAGE INTERPRETED

INTERPRETER NAME

Interpreter Type: ☐ Qualified Medical Interpreter (In-Person) ☐ Language Line / Telephone ☐ Video Remote Interpreter (VRI)

☐ Family Member (document reason qualified interpreter was not used)

INTERPRETER SIGNATURE

_____ Date: _____

End of patient-facing packet. Internal document tracking, release checklist, and final approval records are maintained in the Agency Record Copy only.